

5 May 2023

Lucy Letby - Examination-in-chief day 2: Babies A, B, C, D, E & F

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MS LUCY LETBY (continued)

Examination-in-chief by MR MYERS (continued)

Friday, 5 May 2023
(10.27 am)

(In the absence of the jury)

MR JUSTICE GOSS: Before the jury is brought into court, Mr Myers, Mr Johnson, I hope you've seen -- I'm sorry it's been sent to you late -- the email request from the BBC in relation to the release of copies of some of the exhibits.

MR JOHNSON: I haven't had time -- I have seen them but I --

MR JUSTICE GOSS: Can we come back to it then later?

MR JOHNSON: Of course.

MR JUSTICE GOSS: Mr Myers?

MR MYERS: I've only just seen it, my Lord.

MR JUSTICE GOSS: All right, we'll come back -- if you could perhaps -- this afternoon we'll come back to it. It won't take very long. It's obviously something of which you're aware because there's been discussion and the CPS actually suggested the BBC contact me. That's what it says here.

MR JOHNSON: All right. I've got the gist and I have forwarded it to the person that it needs to go to, so that person I can discuss it.

MR JUSTICE GOSS: Absolutely, yes. It's not urgent.

Thank you.

(In the presence of the jury)

MS LUCY LETBY (continued)

Examination-in-chief by MR MYERS (continued)

MR MYERS: Ms Letby, we're going to go to count 1 on the indictment, which is the case of [Baby A]. What I'll do, as we

go through these counts on the indictment, is just to remind us all, for your benefit and for the benefit of the ladies and gentlemen on the jury, briefly of the details relating to the birth of the baby we're looking at and then, either at the outset or when we come to it at an appropriate place, we'll look at any relevant nursing notes, particularly the ones that are written by you. All right? That's how we'll deal with it. We'll be assisted with that.

There will be an amount of moving between the documents, ladies and gentlemen, like the interviews and the charts, so if you can keep an eye on where your files are and we will take our time to make sure we've got the right material, and you too, Ms Letby.

[Baby A]. I'm going to ask if we can put up tile 1, please. Thank you, Mr Murphy. If we go inside that, please.

We can see the name [Surname of Babies A & B], referred to on the indictment as [Baby A], mother [Mother of Babies A & B].

[Baby A] was born on 7 June 2015. We see elsewhere in fact, but we see that's at 20.31, so just after 8.30 in the evening, on 7 June at 31 weeks and 2 days. He was the second of the twins, twin 1 was his sister, [Baby B], and we can see here his weight is 1,660 grams. So born by caesarean.

Could we go to tile 3, please. We'll go into that.

This sets out those details. Can we go behind that, please? Scroll down if you would, please, Mr Murphy.

[Baby A] was born with antiphospholipid syndrome.

There's the maternal history of high blood pressure and a previous stroke, so medication for that.

The event that we're looking at in count 1 is on the evening of 8 June and we will come to the details shortly. But just to remind us, 8 June. And at 20.05, there was a 10% dextrose infusion that began with [Baby A] via the long line. At 20.26, he collapsed and sadly died at 20.58 that evening.

We'll just look at the notes that deal with that and then we'll move to the questions, Ms Letby. But this is to assist you and everybody. The first note I'm going to go to, with your assistance, Mr Murphy, is the note by Melanie Taylor at tile 224. Could we go into that, please? Can we enlarge the text section on the right-hand side? If you could enlarge first of all the entry on the centre of the right-hand side. Thank you.

I'm going to pick this up from the entry at 19.05, so this is before the shift when you came on, Ms Letby.

8 June, 19.05, by Melanie Taylor. It says:

"UVC in wrong position. Reinserted SHO MacCarrick. Again in wrong position. Cannula tissued. Doctors busy on ward 30. Aware no fluids running for a couple of hours. Long line inserted by Registrar Harkness. Awaiting X-ray. Remains settled on N CPAP. Enteral feeds of donor EBM started at 1ml two-hourly."

That's the note at 19.05. I'm going to go next to the note at 21.28 that follows that and goes over the page. At 21.28, which is after the event, Nurse Taylor put this in:

"Observations stable. Pink, well perfused. UVC still in situ but in wrong position. To be used if no other access available."

Then over the page, if we read on, please:

"Long line inserted by registrar Dr Harkness. Secured, X-rayed. 10% dextrose run through and connected to long line. 10% dextrose run as per Dr Harkness until line confirmed. Care handed over to Lucy Letby at 20.00. Before leaving, apnoea/heart rate dropped. Cardiac compressions and resus commenced. See documentation by Lucy Letby and medical notes."

So we'll turn to that and then we will ask you some questions, Ms Letby. Could we go to tile 228, please, Mr Murphy? These are your notes, Ms Letby, in effect following on.

So we go into that and it starts at the bottom right:

"Written in retrospect."

This a note at 07.56, I should say, on 9 June, so the following morning:

"Care taken over at 20.00. Emergency equipment checked. Fluids calculated. [Baby A] nursed on CPAP."

Details there. If we go over the page, though, please:

"Observations: stable at 20.05. 10% glucose commenced via long line with Staff Nurse Taylor as agreed by Reg Harkness who was present. UVC remained in situ from day shift. Instructed line not to be used by reg. [Baby A] noted to be jittery. Was due to have blood gas and blood sugar taken. At 20.20 [Baby A]'s hands and feet noted to be white, centrally pale and poor perfusion. [Baby A] became apnoeic, Registrar Harkness in the nursery and assistance called for. 10% glucose infusion disconnected

immediately. [Baby A] making nil respiratory effort. Neopuffed with Guedel airway. Shortly after no heart rate detected and full resuscitation commenced as per medical notes. UVC used to administer drugs. Parents and maternal grandmother present. Dr Jayaram in attendance throughout. [Baby A] passed away at 20.58 and was given to parents to cuddle."

I'm going to stop at that point, Ms Letby.

If we need any more detail it's there. That's just to assist us with what we're dealing with with count 1, so I'm going to ask to take that down from the screen for the time being, Mr Murphy. Thank you.

I want to pause before we turn to the events that night and just remind ourselves, just to get some context, we're looking at the middle of 2015, June 2015.

Do you recall where you were living round about this time, Ms Letby?

LETBY: I moved around that time.

MYERS: I wonder if you could put up, to remind us, exhibit J355, please, Mr Murphy. We saw this when we last looked at your evidence. We know the events with [Baby A] are the 8th into 9 June:

"1 June, Ash House."

So what do we know happened on 1 June?

LETBY: That's where I moved to.

MYERS: Right. Hen party in York on 5 June?

LETBY: Yes.

MYERS: Had you settled back into Ash House by the time we're dealing with the events with [Baby A]?

LETBY: No. I was still in the process of moving and unpacking.

MYERS: Right. Do you recall whether you'd been expecting to come in on that evening shift of 8 June?

LETBY: No, I was -- I received a text message from Yvonne Griffiths, the deputy manager, asking me to work that morning.

MYERS: Could we have a look, please, at tile 69, Mr Murphy.

We're going to look at a series of messages starting at tile 69. This is from Yvonne Griffiths to you, 8 June, 9.21 in the morning. So a little under 12 hours before the event we're looking at:

"Morning, Lucy, hope you had a lovely weekend. Just wondering if you can split your nights this week and work tonight and tomorrow and come off Thursday and Friday."

If we go to the following tile, tile 70, you reply at 9.22:

"Yes, that's fine. Is it busy?"

So would you have known you were due to be called in to assist before these messages?

LETBY: No.

MYERS: You've asked if it's busy. Can we go to tile 71, response by Yvonne Griffiths:

"Oh thank you, Lucy. We have three on CPAP, singleton Sat and twins last night, all 30-weekers, so just wanted six staff on, thanks."

Tile 72, please. You now to Jennifer Jones-Key:

"Sorry I was in bed. Hope you slept. I'm working tonight and tomorrow now as busy."

Just pausing there, was it unusual to get requests to come in like this over this period?

LETBY: No, it happened frequently.

MYERS: Were you often requested to do that?

LETBY: Yes.

MYERS: And did you assist?

LETBY: As much as I was able to, yes.

MYERS: Why were you ready to do that?

LETBY: I was very flexible. As I say, I lived locally, I had no commitments outside of work, so I was very happy to help when I could and adapt my shifts as needed.

MYERS: You went in for the evening shift; is that right?

LETBY: Yes.

MYERS: And we can see, and let's remind ourselves, the layout for that. That's on tiles 161 and 162. So can we go to 161, please.

Monday, 8 June at 19.30. Could you just help us, when it's the evening shift, do you regard it as the 19.30 shift or a 20.00 shift? What time do you regard it as starting?

LETBY: 19.30.

MYERS: 19.30?

LETBY: Yes.

MYERS: I'm not going to read out everything that we can see there but we can see who you're on with. If we go to tile 162 to see who is designated with which baby. You were the designated nurse for [Baby A], weren't you?

LETBY: Yes.

MYERS: At this stage of the sequence of events, we don't have the nurseries marked on. That came with the diagrams later, but we know [Baby A] and [Baby B] were both in nursery 1; is that right?

LETBY: That's right.

MYERS: You remember that, do you?

LETBY: Yes.

MYERS: When did you first learn that you'd be caring for [Baby A]?

LETBY: When I arrived on unit that evening and was told at handover that's who I would be allocated to.

MYERS: And do you recall what time you would have got to the unit?

LETBY: I would have been ready to start my shift at 19.30.

MYERS: Do you remember what followed? We've got the notes but do you have any recollection independent of the notes of what followed?

LETBY: Yes, I do, yes.

MYERS: We'll go back over things in the detail we need to but could you start by telling us what happened? You get there that evening for the shift to start. We know that Mel Taylor had been looking after [Baby A].

LETBY: That's right.

MYERS: Can you describe events from the time that you get on to the unit?

LETBY: So I received the group handover that we get at the start of a shift and I knew that I'd been allocated to look after [Baby A] in nursery 1. I went straight into nursery 1 to get handover then from Melanie Taylor. At that point there was a lot of activity going on in the nursery.

MYERS: Pause there for a moment.

LETBY: Yes.

MYERS: You find out then that you're looking after [Baby A]; is that right?

LETBY: That's right.

MYERS: You go into the nursery. Do you recall who was in the nursery or do you not?

LETBY: Yes, so Melanie Taylor was in the nursery and Dr Harkness was in there doing a procedure.

MYERS: When you refer to there being activity in there, is that what you mean or is there something else that sticks in your mind?

LETBY: So I remember there was just a lot going on in the nursery, the lights were up full, I think Dr Harkness was doing a line procedure at the time, and I remember Melanie Taylor was preparing fluids for [Baby A].

MYERS: All right. You went into nursery 1?

LETBY: Yes.

MYERS: What happened once you went into nursery 1?

LETBY: I went into nursery 1 and Melanie Taylor was there, preparing fluids for [Baby A], and she explained that [Baby A] had been without fluids for a few hours because he'd needed a line inserting, and as a priority we got straight on with connecting those fluids to [Baby A].

MYERS: Right. You say [Baby A] -- she explained he'd been without fluids for some hours?

LETBY: She did, yes.

MYERS: Could we look at tile 35, which is the intensive care chart for this period for [Baby A]. If we go behind that.

And if we just start with this, actually, we can see this is for 8 June, Ms Letby, for [Baby A], it says [Baby A]. We're familiar with the charts now so I'm going to ask if we could scroll down. We can see up to 16.00, there are entries, and then at 16.00 that day for [Baby A] it says "cannula tissued".

LETBY: Yes.

MYERS: And apart from an entry for NGT, at 18.00, which is some expressed breast milk via the NGT, what does that tell us, the fact after 16.00 the various readings have stopped?

LETBY: So after that point at 16.00 his peripheral line has tissued which means he can no longer receive IV fluids.

So at that point, from 16.00 onwards, or between 16.00 and 20.00, he didn't have any fluids running.

MYERS: That's what was explained to you?

LETBY: Yes.

MYERS: That had been stopped?

LETBY: Yes.

MYERS: There's something else I would like to ask you about.

It is on tile 145. There's reference to a line that had been positioned. It's at tile 145, please. It's in Dr Harkness' clinical notes. As a general rule I'm not going to be taking you through clinical notes, but where there's something in them I would like you to assist us with I will. If we scroll to the bottom of that page we can see something in Dr Harkness' notes that said "X-ray review". It's a sticker, it says the time of the X-ray. For the date it's got 8 June 15, 19.09. And there's a description of what's there, but under "Comments" it says:

"Long line at right subclavian/SVC [which is superior vena cava]. To be pulled back."

Can you see that?

LETBY: Yes.

MYERS: That's what it was saying at 19.09. Were you aware that there was a repositioning of the long line that was due to take place? Had that been explained to you?

LETBY: I was told at that time that the line was suitable for use.

MYERS: You were told it was suitable for use?

LETBY: Yes.

MYERS: And who told you that?

LETBY: Melanie Taylor and Dr Harkness.

MYERS: Right. Is that the line that the 10% glucose went down?

LETBY: Yes.

MYERS: We can take that down, please, Mr Murphy. Thank you.

Can you explain then what happens next? You say that Melanie Taylor had been getting ready.

LETBY: Yes, when I entered the nursery Melanie Taylor was dressed sterilely, which is what we need to do to prepare fluids. She had the IV fluid trolley out and had the fluids and had gloves and apron on and was starting to run through some fluids into the line. And when I arrived in the nursery, we decided a this that would be our priority, we would both check the fluids, get those running on [Baby A] and then have a formal handover.

MYERS: So we know you get on to the unit round about, not precisely, 7.30.

LETBY: Yes.

MYERS: If it's possible, how far into getting on to the unit would this conversation and these arrangements start with [Baby A]?

LETBY: First we would have had collective handover at the nurses' station before that, so that would have taken 20 minutes or so, maybe, even more.

MYERS: Then you go to the nursery?

LETBY: That's right.

MYERS: And then when you're in the nursery, that's where you begin to make arrangements with Melanie Taylor?

LETBY: Yes.

MYERS: Right. Can you tell us who was going to do what with regard to running fluids through the line?

LETBY: So when you're running fluids through a long line one member of staff has to be sterile and that person was Melanie Taylor.

MYERS: What does that mean when you say sterile? What have they done or how are they that means they are the sterile nurse?

LETBY: So that nurse is wearing gloves and an apron. They have sterilised the whole of the area that we're working with. We have a fluids trolley, they would have sterilised the area. And then any of the fluids that they're touching, the connections that go to it, that is all kept sterile.

MYERS: We've heard it involves two nurses for this. What's the other nurse doing?

LETBY: So you have to have a role then of a dirty nurse, which is the person who does the things such as connecting it to the pump and doing any of the practical things that that person can't touch because they're sterile.

MYERS: So who was going to be doing that when it came too [Baby A] and the dextrose?

LETBY: I was the one that was the dirty nurse that would be programming the pump.

MYERS: Was it a premade bag of dextrose that was used?

LETBY: Yes, it was.

MYERS: Where would that have come from?

LETBY: So Mel had it out when I arrived, but the bags are kept in nursery 1 in the cupboard.

MYERS: It was already out when you got there?

LETBY: Yes, it was. It was all on the trolley ready.

MYERS: So the bag's ready, Melanie Taylor was the sterile nurse, you were going to assist. What happened next?

LETBY: So we went to [Baby A]'s cot side. Mel finished connecting the fluids and drawing them through the line.

She then went to [Baby A] to attach the fluids and I turned my attention then to hanging the bag and programming the pump.

MYERS: Right. So can you give us a little more detail? Who was doing what with which bit of the equipment?

LETBY: At that time Mel is the one who is handling the bag of 10% and the infusion line that runs with that. She was then the one that would physically go to [Baby A], clean the long line and attach that to the line.

MYERS: What does that involve, attaching it to the line, just so we can follow? What actually does that mean?

LETBY: You have to access the line, you clean the line with an alcohol wipe to make the area sterile. She would then have needed to flush the line to check that it was patent. And after that then you would connect the fluid line to the end of the long line.

MYERS: I can imagine there's a line coming down from the bag and the apparatus there.

LETBY: Yes.

MYERS: When you're talking about connecting it, what is it being connected to so far as [Baby A] is concerned?

LETBY: It'd be connected to the port that's coming out of the long line that's in [Baby A].

MYERS: So the line that had been positioned by Dr Harkness?

LETBY: Yes.

MYERS: Do you remember where it was coming out of [Baby A], where the port was?

LETBY: His left arm.

MYERS: So that's what's being connected?

LETBY: Yes.

MYERS: And who's doing that connecting?

LETBY: Melanie Taylor because she's the one who's sterile.

MYERS: And then could you repeat, what is it that you were doing whilst she did that?

LETBY: So I was responsible for hanging the 10% bags, that's putting it onto the drip stand next to the cot side, and then programming the pump.

MYERS: And is that what happened?

LETBY: Yes.

MYERS: And you mentioned about the line, did you say, being flushed to make sure it's patent?

LETBY: Yes, the usual practice is whenever you are attaching something to any line you would flush the line with sodium chloride first to check that the line is working.

MYERS: That's usual -- that's standard practice?

LETBY: That's standard practice, really, for anything going through a line, yes.

MYERS: And did that happen?

LETBY: I can't -- I wasn't watching Mel specifically but that is usual practice, yes.

MYERS: If we please look at tile 174, which is the IV prescription chart. Help us with the time. Go behind that, please, Mr Murphy.

We can see here:

"8 June, 10% dextrose, 500ml bag."

And it's got a rate there. "Route" -- what does it say under "route"?

LETBY: "Long line."

MYERS: It's been prescribed by or which registrar?

LETBY: Dr Harkness.

MYERS: And then it says "given by". Who's the -- is it your signature there?

LETBY: Yes, my signature's on the bottom.

MYERS: You say Melanie Taylor also gave it?

LETBY: Yes.

MYERS: So that's the other signature?

LETBY: Yes.

MYERS: Let me just ask you this because others have been asked it: is there a clear significance to whose name goes where as to above or under when it's a signature?

LETBY: No, not at all, no.

MYERS: It says "Date and time started". It looks like -- can you tell us what that says?

LETBY: I say that's 20.05.

MYERS: So five past eight in the evening?

LETBY: Yes.

MYERS: And is it you that wrote that in or Mel or someone else?

Do you know?

LETBY: It would have been myself or Mel. I can't say.

MYERS: One or other?

LETBY: Yes.

MYERS: Do you remember who?

LETBY: No.

MYERS: And what does that tell us?

LETBY: That is the exact time that the fluids began to run through the pump.

MYERS: All right. Is that what took place?

LETBY: Yes.

MYERS: So 10% dextrose begins to go down the line?

LETBY: Yes, we would only sign for it once we were happy that the fluid was actually infusing through the line.

MYERS: All right. So --

LETBY: So at this point it has been connected and the pump is running.

MYERS: All right. So at that point, if you can remember, just tell us what happens next.

LETBY: As in what did I do next?

MYERS: Yes, leading up to the time when anything was noticed about [Baby A], can you recall anything that took place between this and what happened round about 20.26?

LETBY: Yes, so once we'd sorted out the fluids I then had a handover from Mel, and then she went to the computer station to start writing her notes and I then started doing my equipment checks.

MYERS: Pausing there, we know there's a handover at the beginning of the shift. When you went to nursery 1, you then assisted Melanie Taylor with [Baby A] and the dextrose; is that right?

LETBY: That was the first thing we did, yes.

MYERS: The first thing you did. So had you actually had the cot side handover discussion before you did the fluid?

LETBY: No.

MYERS: Right. So you say that's what you then had after the fluid was running?

LETBY: That's right.

MYERS: And then Mel went to deal with her own business and you said you were doing some checks?

LETBY: Yes.

MYERS: What checks would they be?

LETBY: At the start of every shift we do equipment checks, so that's checking all of the equipment around the cot side, so the emergency equipment, the oxygen, the suction points, any equipment that's around the baby, basically, we are checking, and that's the standard thing at the start of every shift.

MYERS: Do you recall where Dr Harkness was during this?

LETBY: Yes, Dr Harkness was with [Baby B], I believe.

MYERS: In the same nursery?

LETBY: Yes, the lights were up and he was doing a procedure and I think it was on [Baby B], and Caroline Bennion was there as well.

MYERS: What was the first indication you had that anything might not have been right?

LETBY: I noticed [Baby A] to be jittery when I was doing my observations.

MYERS: Let's if we could put up tile 228 again, please. It's the second page of tile 228. If we look across to the left-hand side a couple of lines down. We've got -- it refers to 10% glucose, after a time of 20.05, has been given. It refers to Nurse Taylor and Dr Harkness and that the UVC remained in situ. Then it says this on the fourth line down:

"[Baby A] noted to be jittery. Was due to have blood gas and blood sugar taken."

Why did you make a note -- when you did this retrospectively, and these notes are written at 7.56 the next morning, why did you make a note, "Noted to be jittery"?

LETBY: Jittery is an abnormal finding. That is not something that we want a baby to be displaying.

MYERS: What does jittery mean in case anybody can't really follow what that describes?

LETBY: It's like an involuntary jerking movement of the limbs.

MYERS: Do you recall now, casting your mind back, how pronounced that was or how subtle it was? You describe it to us if you can remember.

LETBY: I remember it was noticeable enough that whilst I was just doing my equipment checks I could see he was visibly jittery.

MYERS: Tell us what happened next then.

LETBY: So whilst I was carrying out my equipment checks around the bedside, [Baby A]'s monitor sounded and that's when we noted that his colour had changed and he was apnoeic.

MYERS: Monitor, just help us, what's the purpose of the monitor that sounded?

LETBY: So the babies in nursery 1 are on a Philips monitor, which is full intensive monitoring, at all times and if any values go out of the recommended reference range, then an alarm will sound.

MYERS: Is that the alarm you heard?

LETBY: Yes.

MYERS: Do you know what that alarm was indicating?

LETBY: I don't recall what the alarm was indicating at that time, no.

MYERS: We've got the note here, your note, but, you describe to us, was there anything about [Baby A] that you noticed when you went to him?

LETBY: Yes, the most obvious thing I noticed was that he was -- his hands and feet were white.

MYERS: Can you describe to us what happened when the alarm had gone and you'd noticed this?

LETBY: I went over to [Baby A]. He wasn't breathing, so we began to Neopuff him. Mel Taylor came straight over, as did Dave Harkness, and because his limbs had gone white the first thing we did was disconnect that bag of 10% fluid from the line.

MYERS: Just pause there, please. When you say "disconnect the bag of 10% fluid", that's the bag going down the long line that had been positioned earlier by Dr Harkness?

LETBY: That's right.

MYERS: And the first thing that happened was to disconnect that?

LETBY: Yes, so Dr Harkness advised that we were to stop the fluids immediately and (overspeaking) --

MYERS: That was his advice?

LETBY: It was, yes.

MYERS: In the description in, the note you've talked about:

"Hands and feet noted to be white."

And you put:

"Centrally pale and poor perfusion."

So there can be no doubt, what do you mean by "centrally pale and poor perfusion"? How does that look on the baby as a whole?

LETBY: So his limbs were white and centrally he was pale, but not as white as his limbs.

MYERS: By centrally, just tell us, which part of the body?

LETBY: With centrally I'm talking about the torso and abdomen.

MYERS: Right. And "poor perfusion" what does that mean?

LETBY: Poor perfusion is a reflection there that obviously he's pale and white is a sign of poor perfusion.

MYERS: Do you recall how his breathing was at this point?

LETBY: He was apnoeic.

MYERS: Meaning?

LETBY: So he was not breathing at that point.

MYERS: Please talk us through what happens after that. You're all there now, you, Dr Harkness and Melanie Taylor.

LETBY: And Caroline Bennion.

MYERS: Do you remember where she had been beforehand?

LETBY: She was the allocated nurse to [Baby B], so I believe she was helping Dr Harkness with what was going on at that point.

MYERS: So where would she have been in fact?

LETBY: In the nursery at [Baby B]'s cot side.

MYERS: In nursery 1?

LETBY: Yes.

MYERS: Do you recall how long she'd been there for?

LETBY: She was there the whole time because she was receiving handover for [Baby B].

MYERS: Let's just pause there because so far you have given us the name of Dr Harkness as being present?

LETBY: Yes.

MYERS: Nurse Taylor?

LETBY: Yes.

MYERS: Because she'd done the handover to you?

LETBY: And Nurse Taylor was also handing over to Caroline Bennion because she'd had [Baby B] that day as well.

MYERS: So Caroline Bennion was also in that nursery too?

LETBY: She was, yes.

MYERS: And had been during handover?

LETBY: Yes.

MYERS: Describe what happened next then, please.

LETBY: So once I noticed that [Baby A] was apnoeic, I began the usual procedure of starting to Neopuff him.

Dr Harkness, as I say, came straight over. One of us disconnected the fluids, I'm not sure who that was, and after that [Baby A] just continued to deteriorate and we then lost his heart rate --

MYERS: Right.

LETBY: -- and an emergency crash call was then put out.

MYERS: And a crash call, again just help us with this, I won't keep going back to these things, but so we're clear what you mean by these things as we go through your evidence, by an emergency crash call what does that actually involve happening?

LETBY: So an emergency crash call is where we -- we call it four 2s, it's the number on the phone, it's 2222, and that goes straight through to the emergency line on the switchboard and that's how we'd then request that we need the neonatal team to attend urgently.

MYERS: Is it a doctor who then attends?

LETBY: It is, yes. That would go out to the SHO, the registrar and consultant if needed.

MYERS: Do you recall who did attend?

LETBY: Yes. I remember Dr Jayaram came very quickly.

MYERS: Were there any other people who came to assist with what was taking place?

LETBY: [Nurse A].

MYERS: Do you recall when it was that she came?

LETBY: She came very soon after we'd noticed the change in [Baby A].

MYERS: Do you know where she'd been beforehand?

LETBY: No.

MYERS: Do you recall the events of the resuscitation that took place? We can take the tile down, please.

Do you recall the events of the resuscitation that took place clearly?

LETBY: No, not in any great detail, no.

MYERS: Do you recall how you were feeling, if you have any recollection of this, as this resuscitation unfolded?

LETBY: I remember it just being a huge, unexpected shock. We just -- it felt we'd literally just walked through the door of the shift and this was happening.

MYERS: Do you recall whether [Baby A]'s parents were present at any stage after this had started?

LETBY: Yes, they were called for when [Baby A] deteriorated, yes.

MYERS: Which nurse assisted the parents with what was happening?

LETBY: I'm not sure specifically.

MYERS: We know that, sadly, the resuscitation wasn't successful and we know that [Baby A] died shortly before 9 o'clock that evening.

Once [Baby A] had died, whose role was it to assist the parents with whatever followed?

LETBY: It was myself as allocated nurse.

MYERS: Did you assist them?

LETBY: Yes.

MYERS: Do you recall what you had to do or what you did?

LETBY: I recall doing hand and footprints for [Baby A] because the parents also wanted them done of [Baby B] as well.

Other than that, I can't remember specific details.

MYERS: In fact, the bereavement checklist, which we looked at on Tuesday, related to [Baby A]. I'm not going to go back to it now, but does that set out the sort of things that you did?

LETBY: Yes.

MYERS: Was there any other nurse or nurses who helped you as you were dealing with that?

LETBY: Yes. [Nurse A] helped me with the hand and footprints. That's something that needs to be done by two people.

MYERS: Do you recall whether or not there was a memory box that was put together for [Baby A]'s mum and dad?

LETBY: Yes, there was.

MYERS: Who did that?

LETBY: Myself -- I started it, yes.

MYERS: Do you recall whether or not there was a baptism or any religious or spiritual assistance offered to the parent?

LETBY: Yes. When [Baby A] was being resuscitated, a baptism was offered to the family, which they accepted, and both [Baby B] and [Baby A] were baptised together.

MYERS: Is the offering of a baptism -- at this point was the offering of a baptism or other spiritual rite or religious rite something that was part of the practice when a situation like this emerged?

LETBY: Yes, that's right.

MYERS: Is there anything that you did so far as the medical equipment was concerned after [Baby A] had died?

LETBY: So after [Baby A] had died, I felt that we should retain the bag of fluids and the line that was running through -- that was running those fluids through.

MYERS: Which fluids are you referring to when you say "the bag of fluids"?

LETBY: The bag that Melanie Taylor and I had put up, the bag of 10% dextrose and the infusion line connected to that.

So I labelled the bag and the line that was attached to it and then put it into the sluice room.

MYERS: And why did do you that?

LETBY: At the time it was discussed that there'd been this sort of connection with -- we had connected the fluids and then [Baby A] deteriorated and it was felt at the time that we should be checking everything related to the line and the fluids. So we were keeping the bag to ensure that it could be tested or whatever might be needed to be done with it if needed.

MYERS: And did you have any involvement with the bag after that?

LETBY: No.

MYERS: Or do you know what happened to it after that?

LETBY: No.

MYERS: We've seen, after this happened, that at various points you were in contact with your colleagues, talking about what had taken place on the messaging.

LETBY: Yes.

MYERS: Why did you contact people to talk about what had taken place?

LETBY: They were my support system. We were a close team and things like that is something you can only really discuss with somebody who is in the profession.

MYERS: How did you feel with what had happened with [Baby A]?

How did it affect you?

LETBY: Just stunned. It was complete shock for all of us and, yeah, it was just complete -- it felt like we'd walked through the door into this awful situation and that was obviously the first time I'd met [Baby A], the first time I'd met the parents. It was a huge shock.

MYERS: Just with regard to the messaging, let's have a look at a little of it. I won't keep on going to all of it.

Tile 249, please. 248 first, please, Mr Murphy.

It's a message from [Nurse A] to you and the following day at 18.38. She said:

"You did amazing. So proud of you."

In fact, just pausing there, the following day were you back on a shift on the following night shift?

LETBY: Yes.

MYERS: Who were you looking after on that shift; do you recall?

LETBY: No.

MYERS: But that's the shift when we turn to events with [Baby B], isn't it?

LETBY: It is, yes.

MYERS: Just to keep track of everything. She said:

"You did amazing, so proud of you."

Then 249, please. Seconds later:

"Hope that doesn't sound patronising. It's genuine. You did fab."

And kisses. Then, please, 250. Now from you to [Nurse A] at 18.39:

"I think it's Kate W instead of Caroline. I think it'd be nice for you to have [Baby B]."

And then 251 is where you actually respond to what [Nurse A] has said to you:

"No, it's not patronising at all. Appreciate you saying that and thanks for letting me do it but supporting me so well."

Was there anything odd in your opinion in exchanging messages like this?

LETBY: No, this would happen frequently.

MYERS: When you say frequently, with reference to what?

LETBY: So I would frequently message various staff members about different babies on the unit or about personal and social things.

MYERS: How important was the support that you got from colleagues when dealing with difficult situations on the unit?

LETBY: It was hugely -- that was my main source of support, living alone and with there being no formal process in the workplace to support anyone, my colleagues were my main support, yes.

MYERS: And was it also just them giving support to you or did you ever give support to people?

LETBY: Both.

MYERS: In your experience was it unusual for colleagues to be messaging one another after something had happened that may carry some impact?

LETBY: No, not at all. I'd say it's something we would all do.

We were all regularly in touch outside of work.

MYERS: Now, also after the events of that night one of the things we've seen is there's a Facebook search for [Mother of Babies A & B], [Baby A]'s mother. I'll give you the reference for that. Tile 231, please, Mr Murphy.

The following morning, there'd been a Facebook search for [Mother of Babies A & B]. Can you explain to us why you'd done that search?

LETBY: To walk into the unit that evening and for the first time to have met those parents, be in that awful situation, I think it was just curiosity that I wanted to see the people behind the awful event that had happened. I think it was just curiosity. They were on my mind.

MYERS: We know there are later searches on 10 June in the evening. They are amongst other searches that are ongoing, but on 10 June on the evening, you search for [Mother of Babies A & B] and then on 25 June you search for her.

Again, why would you look for [Mother of Babies A & B]? I don't ask critically, but if you could just explain to the jury so they can understand it, as much as you can explain why you would look for her.

LETBY: It's a common pattern of behaviour for me. If I think of somebody, I would look them up. [Baby A] and [Baby B] were on my mind quite a bit at that time.

MYERS: And when you did searches, did you search for people in isolation or would you be performing a number of searches at the same time?

LETBY: No, it would usually be done in quick succession, multiple people over a short period of time.

MYERS: Are the only people that you looked up parents of babies on this indictment?

LETBY: No.

MYERS: Was there any kind of debrief held after [Baby A] had died, an official debrief on the unit?

LETBY: There was a debrief, I believe, a few days later.

MYERS: Right. Do you recall who led that?

LETBY: Dr Jayaram.

MYERS: Do you recall what the purpose of that debrief was?

LETBY: So it was to just look at the sort of the resus itself and to see if there was anything obvious that we needed to learn from at that point.

MYERS: The debrief that takes place, is there any kind of discussion amongst staff more informally in terms of support or any type of emotional support for what people have been through?

LETBY: No, it's more clinically based.

MYERS: Yes. Did it affect you, going through something like this?

LETBY: Yes, it did, yes.

MYERS: It's being alleged, of course, that you did this. Did you?

LETBY: No.

MYERS: What's it like to have that allegation made?

LETBY: It's awful. Obviously, that day I wasn't even supposed to be working that night and I just... It's -- yeah, it was just such a shock to walk into that situation.

MYERS: How lasting is the impact on the people who are involved in a situation like that?

LETBY: You never forget something like that.

MYERS: I'm going to turn next to the case of [Baby B].

Again, I won't repeat this each time we move between cases, but if I move from one case to the next there's no insensitivity or lack of appreciation of the enormity of what we're dealing with. That goes for you as well, Ms Letby, does it?

LETBY: Yes.

MYERS: We have to move from case to case to look at the evidence and, where we can, we'll move from one to the next unless, with his Lordship's leave, there's a reason for a break. But we're a little early for that.

MR JUSTICE GOSS: It's too early, we'll carry on.

MR MYERS: So just pause, keeping in mind the gravity of what we're dealing with, but looking at the evidence of what comes next, which is count 2 on this indictment, ladies and gentlemen, the case of [Baby B], [Baby A]'s sister.

Again, what we'll do is just orientate ourselves to what happened by some of the basic facts. [Baby B] is the twin sister of [Baby A]. The incident relating to [Baby B] happened the following evening. Let's start with tile 1, please, Mr Murphy, which is just the details of [Baby B]'s birth.

Can we go behind that, please. Thank you. Count 2, [Baby B]. Born on 7 June 2015 at 20.30. It says here:

"31 plus 2 weeks, twin 1."

Her weight is not recorded here, but her weight, no dispute, is 1,669 grams. Caesarean, of course, as her brother was.

If we scroll down a little bit, please, to the text giving details of the pregnancy. Again, antiphospholipid syndrome and

the reference to warfarin and pregnancy and the medication given there. That's for high blood pressure and a previous stroke.

Can we go next to tile 2, please, which just deals with some events soon after [Baby B] had been born. If we go into that, please. If we scroll down, please, and keep scrolling.

These are some notes by ST1 Thomas. I just want to identify a few matters here and then we'll move forwards. These notes are made at 21.00. They set out the staff present at the delivery and what happened.

And then they have the following:

"Brought to Resuscitaire. Wrapped in plastic bag and hat placed on head. Face stimulated. No response. Blue and floppy. Poor tone. Heart rate approximately 50. Five inflation breaths given. No chest movement noted and no response in heart rate. Airway repositioned."

Does that say "two-man airway"?

LETBY: Yes, so it's two people are needed to manage the airway.

MYERS: "Two-man airway plus five inflation breaths given. No clear chest movement and heart rate remained below 60 with sats of 40%. Airway repositioned with shoulder roll. Five inflation breaths given and chest movement seen."

Over the page, please, if it continues:

"Some attempts at resps. No change in heart rate, around 50. IPPV..."

That's independent positive pressure ventilation; is that right?

LETBY: Yes.

MYERS: "... given for approximately 30 seconds and chest movements good. Oxygen up to 40%. Heart rate less than 60 so CPR commenced between 2 and 3 minutes of age.

Neonatal team requested. Neonatal consultant called at 5 minutes. No respiratory effort. First attempt at intubation. Tube appeared to pass cords. Unclear chest movement. No colour change on capnograph. No increase in heart rate."

In fact, after that and further attempts, the intubation became successful and [Baby B] got the breathing support that she needed. But that's just dealing with the process from birth and the early part of [Baby B]'s life.

That all takes place on the evening of 7 June after she'd been born. And before we actually look at the events on the unit that we're looking at in this count, can we just look at the nursing note of [Nurse A], which deals with the event itself so we can keep that in mind when we hear your evidence.

That's at tile 218, please. Just to remind us all, we can go into that.

We'll look at the detail in a moment in the note, but just to remind us with the headings. This event, the second charge on the indictment, relates to events on 9/6 going into the early morning of 10/6. And as everyone may recall, there's two matters which come up.

One is just before midnight, there was a desaturation and it appeared that [Baby B]'s CPAP prongs had dislodged from her nose. And then round about half past midnight, going into the 10th, there was a sudden desaturation to 50% and skin discolouration was noted. So that's what we're dealing with. We can look at the note now made by [Nurse A].

We're going to look at the note that says 10 June 2015 at 07.28. This is a note by [Nurse A] the following morning:

"Shortly before midnight [Baby B] desaturated to 75%. Found to have pushed CPAP prongs out of nose. Prongs and head repositioned. Took a little while and O2 to recover."

That's oxygen:

"Heart rate remained stable, good respiratory effort throughout. Once settled, CBG taken..."

That's capillary blood gas, isn't it?

LETBY: Yes.

MYERS: "... which was normal. Dr Lambie on unit and satisfied that [Baby B] was stable."

Can we go to the next entry that follows soon after this one, this entry made at 07.29:

"00.30. Sudden desaturation to 50%. Cyanosed in appearance. Centrally shut down, limp, apnoeic. CMV via Neopuff."

What's CMV mean, can you help with us that?

LETBY: That's continuous mechanical ventilation. It's another phrase of (sic) IPPV that we talked about earlier.

MYERS: All right:

"CMV via Neopuff commenced and chest movement seen. Colour change rapidly to purple blotchiness with white patches. Started to become bradycardic."

Which is a slowing of the heart rate, is it?

LETBY: Yes.

MYERS: "Emergency call for doctors put out. Continued with Neopuff via Guedel airway until Dr Lambie arrived.

Became bradycardic to 80s. Successfully intubated by Dr Lambie and heart rate improved quickly. 0.9 saline bolus given and colour started to improve almost as quickly as it had deteriorated. Started to breathe for self. Morphine commenced and placed on ventilator."

I'll stop there, but we know after that there's a recovery with [Baby B]. So that's what we're dealing with.

Let's just have a look first then, as a starting point, with the layout, who was on duty on that shift and who was looking after which baby. That's on tiles 145 and 146. Can we go to tile 145, Mr Murphy?

How good is your recollection of the details of this shift?

LETBY: I don't have much recollection.

MYERS: Or of what happened with [Baby B]?

LETBY: No.

MYERS: Could we go to tile 146, please, showing the actual layout of the nurseries? Thank you.

We can see in nursery 1 is [Nurse A], designated with [Baby B] and also baby JE. You, Ms Letby, were in nursery 3, designated with EB and HT.

We can see who else is on the shift there. That's just to orientate us to this.

But without a display like this, would you have any chance of remembering who was where doing what?

LETBY: No.

MYERS: Thank you, Mr Murphy, we can take that down, please.

Let's go through events then on that evening. To what extent, as a general rule, do nurses assist one another on a unit looking after the babies?

LETBY: Very much so. A lot of tasks that we do require two people.

MYERS: And will a nurse -- how do you know if a nurse needs help? I don't mean help because of an emergency, I just mean help with a task like prescriptions or with the line.

LETBY: They would come and ask and ask if anybody was free to assist them at that time.

MYERS: We've seen that [Nurse A] made reference to the prongs moving -- dislodging was the way she put it -- and that:

"[Baby B] had desaturated to 75%, found to have pushed CPAP prongs out of nose."

Can prongs be dislodged by babies' movements?

LETBY: Yes, quite easily. It happens frequently.

MYERS: Is there anything unusual about that?

LETBY: No.

MYERS: Do you have any awareness of what went on with [Baby B] with prongs moving?

LETBY: No, I've no recollection of this event.

MYERS: What about with what happened at 00.30? Do you recall any particular involvement with [Baby B] leading up to that?

LETBY: I recall running a blood gas at some point, I think it was about quarter past 12.

MYERS: Let's have a look. Before we get to that, actually, could we put up, please, tile 213? Just go into that, please. Scroll down, please. It's on the second page that we need to go to. Thanks.

Pausing here, can you see it says, "Day 2, 8/6/15"?

It's the second --

LETBY: Yes.

MYERS: -- day down. If we look across, I would like you to assist us with what has happened at this time.

LETBY: On 8 June?

MYERS: Yes.

LETBY: Yes, so on 8 June, at 23.00 hours, TPN and lipids have been started by those two members of staff that have signed there.

MYERS: Yes. Did you have any involvement with [Baby B], as far as you can recall, shortly after midnight on 10 June?

LETBY: Not from memory, no, other than the blood gas.

MYERS: Can we go then to tile 20, which is the blood gas, and go behind that.

If we scroll down, you tell us, where is the blood gas that you're referring to? We'll need to move down to where we have entries for 10 June.

LETBY: It's there, the first line on the 10th June at 00.16.

MYERS: 00.16. Is that your signature at the end of that line?

LETBY: It is, yes.

MYERS: What will you have done for that entry to be there?

What did you do that led to that entry being made?

LETBY: It's likely that I was the person that took the blood sample to the blood gas machine on the unit and analysed the sample and brought the printout back and filled in this chart.

MYERS: Would [Baby B] have been on her own when you took the blood gas sample away to have it analysed?

LETBY: No, no.

MYERS: Do you have any idea who would have been with her?

LETBY: I think it was [Nurse A].

MYERS: You hadn't been working in nursery 1 that evening, had you, you were in a different nursery?

LETBY: That's right.

MYERS: Do you have any idea how [Nurse A] came to ask you for assistance with blood gas?

LETBY: No. It's usual practice that it would take two people, so she may have just asked if I was free at that moment.

MYERS: Right. Apart from that, do you have any independent recollection of being involved with [Baby B] in any way before her deterioration at about 00.30?

LETBY: No.

MYERS: You were, as it happens, questioned in your interview about morphine that was given to [Baby B]. I just want to deal with that. We will be going to the interviews from time to time, ladies and gentlemen, so let's start as we mean to go on. If you track down interview bundle 1, I'll show you the reference.

Do you have a copy of the interview bundles there, Ms Letby?

LETBY: Yes.

MYERS: If we go behind the [Initials of Baby B] divider. Where the initials for the baby are, that's also interview 1 always, and then we've got 2 and 3. So it's [Initials of Baby B], interview 1. If we go forwards to page 12, please. We'll look at this questioning and look at the details for what happened with the morphine.

We'll be going back to this bundle, ladies and gentlemen, during today and this part of the evidence, so keep an eye on it, it'll make it easier to look up the references.

I just want to read a section.

Can you see there's some blue text, Ms Letby?

LETBY: Yes.

MYERS: You were being asked questions about contact that you had had with [Baby B] and the police are looking at the question of what happened and what could have been involved with the collapse that she had. You're shown tile 241, which we'll look at in a moment -- or rather asked about the entry on that document. You said:

"Answer: Okay, that's a morphine bolus that would seem -- I have given that to [Baby B].

"Question: Physically what do you do?

"Answer: You attach the morphine syringe to [Baby B] and give the volume. It says IV. I'm not sure which type of access [Baby B] had, whether it's peripheral, cannula, a long line or a UVC.

"Question: So that's a physical pressing of a syringe into the baby via something?

"Answer: Yes.

"Question: But you can't be sure what that was, what the apparatus was?

"Answer: I'm not sure which line it was delivered through, no.

"Question: Okay. So that and that -- so that will be physical contact?

"Answer: Yes to attach to the line, yes."

And that's right, isn't it, using morphine like that would be physical contact with the baby in that way?

LETBY: Yes, it would, yes.

MYERS: But I'd just like to be clear about the time this actually happened, the morphine. Could we put up tile 241, Mr Murphy, which is the document we're referring to?

If we have a look at that. This is the morphine that's given. Looking at the entry, what's set out there, who is involved in the giving of this morphine to [Baby B]?

LETBY: It's myself and Mary Griffith.

MYERS: Yes. Now of course, the officer was asking about contact in the context of establishing anything that could have been done to cause [Baby B] to deteriorate.

LETBY: Yes.

MYERS: But so we can be clear, what time was it that the morphine was given?

LETBY: So this is at 01.10, and this is as a result of her being intubated.

MYERS: Right. So this is something that happens actually 40 minutes after the collapse, isn't it?

LETBY: That's right.

MYERS: Because of the collapse?

LETBY: It is, yes.

MYERS: So whatever you're being asked about physical collapse, that's not before the collapse at all, is it?

LETBY: No.

MR MYERS: My Lord, can I continue with this, if Ms Letby is able to continue, and perhaps we can conclude --

MR JUSTICE GOSS: Certainly, you just choose the moment.

MR MYERS: Are you all right just to conclude with this?

LETBY: Yes.

MYERS: Do you recall events leading up to [Baby B]'s deterioration at 00.30?

LETBY: Not with any clarity, no.

MYERS: What's the first -- we can take that down, by the way, Mr Murphy. Thank you.

What's the first thing you remember about what happened with [Baby B] that night, apart from possibly the blood gas?

LETBY: I know that there was a deterioration at some point fairly soon after this.

MYERS: Right. What do you remember that you saw or took part in? Could you just help us with what your memory stretches to? Because we don't have the notes from you, the notes are from [Nurse A].

LETBY: Yes.

MYERS: Tell us what you recall.

LETBY: From memory, [Nurse A] and I were both in nursery 1 and at some point we became aware that [Baby B] had deteriorated and her colour had changed.

MYERS: Right.

LETBY: I think [Nurse A] alerted me to that.

MYERS: Do you recall how [Baby B] was? When you say her colour had changed, could you describe that?

LETBY: She'd become quite mottled and dark.

MYERS: All over her body or in particular parts of her body?

LETBY: All over, from memory.

MYERS: Can you describe that colour again?

LETBY: It was a dark mottling colour.

MYERS: A dark mottling colour?

LETBY: Yes.

MYERS: You saw that?

LETBY: Yes.

MYERS: Had you seen anything like that colouring before?

LETBY: What do you mean?

MYERS: Had you seen that type of colouring on any baby before this point?

LETBY: Yes, it was like general mottling that we do see on babies.

MYERS: Right. Was it unusual or was it not unusual in your opinion?

LETBY: It was not unusual, but obviously we were concerned for [Baby B] because of [Baby A]'s decline the night before.

MYERS: You described -- [Baby A], you say, was "centrally pale with poor perfusion".

LETBY: Yes.

MYERS: So did it look the same as [Baby A]'s or different from [Baby A]'s?

LETBY: No, so [Baby A] was pale, he was white. [Baby B] had more colour. It was a purple mottling colour.

MYERS: All right. Do you recall who was with you when you saw that?

LETBY: [Nurse A].

MYERS: Were you joined by anybody else, Ms Letby?

LETBY: Dr Lambie.

MYERS: Is that Rachel Lambie?

LETBY: Yes.

MYERS: Was there any action that you took once this deterioration had been identified?

LETBY: Yes, so at that point I was asked to go round to get the unit camera to take a picture of the colour change in [Baby B].

MYERS: The unit has a camera, does it?

LETBY: It does have a camera, yes.

MYERS: So if anybody sees anything they regard as significant, are they able to access that camera?

LETBY: Yes.

MYERS: Where's it kept?

LETBY: It's kept in the manager's office at the back of the unit.

MYERS: So you fetched the camera?

LETBY: Yes.

MYERS: And what happened after you'd got the camera?

LETBY: So on my return [Baby B] had stabilised and her colouring had returned to normal. So there wasn't anything to photograph.

MYERS: Did you have the camera with you?

LETBY: Yes.

MYERS: And you brought it back?

LETBY: Yes, and I got it very quickly.

MYERS: And can anybody use that camera?

LETBY: Yes.

MYERS: So could someone have taken it off you if they wanted and taken a photograph?

LETBY: Yes.

MYERS: Did anybody?

LETBY: No.

MYERS: Again, could you explain to us why was no photograph taken even though you got the camera?

LETBY: When I returned her colouring was normal, there was no abnormal change then.

MYERS: Pausing for a minute, and I am sorry if you did say this, but I just want to be clear, what was the first -- just going back a short period, what was the first that you knew or the first indication there was a problem with [Baby B]? If you can remember, what brought that to your attention?

LETBY: From my memory it was [Nurse A] alerted me.

MYERS: By doing what?

LETBY: I think she called me over --

MYERS: Right.

LETBY: -- to [Baby B].

MYERS: Do you recall whether you had any part to play in the resuscitation or the stabilisation of [Baby B]?

LETBY: Not from memory, no.

MYERS: Did you have any involvement with her soon after the collapse apart from collecting the camera?

LETBY: So I believe I administered some of the drugs that she needed after intubation.

MYERS: If we just go back, please, to tile 20, which is the blood gas tile. Could we go there, Mr Murphy? Can we go into that, please? We'll scroll down.

Can you see 00.51?

LETBY: Yes.

MYERS: There's a capillary sample taken. Who's taken that sample?

LETBY: I can't say who's taken the sample but that is my writing that's documented that they've analysed it.

MYERS: Again, why can you not say who's taken the sample from this?

LETBY: Because we can never definitively say who has taken a sample. That is not documented. But the likelihood is the person who's documented is not the person who's physically taken the blood sample.

MYERS: As you've explained, the nurse treating might do that and hand the ticket to another nurse to get the analysis performed on the machine?

LETBY: Yes, and then they would remain with the baby.

MYERS: And the other nurse comes back and writes the details up if that's --

LETBY: That's right.

MYERS: -- being done that way? All right.

MR JUSTICE GOSS: Sorry, hand the what to the other nurse?

MR MYERS: Hand the sample.

MR JUSTICE GOSS: You said ticket. That's why I'm querying it.

MR MYERS: Sorry, it's coming back with the ticket.

It's the sample that's handed and then the ticket comes back; is that right?

LETBY: The sample would be handed to the person who then goes to the gas machine and obtains the printout that we've seen, which is then written on to the chart.

MYERS: That's why the nurse writing it down, you say, may not be the nurse who's actually taken the sample?

LETBY: That's right.

MYERS: That was at just before -- just after 00.50, so about 20 minutes after the deterioration; is that right?

LETBY: Yes.

MYERS: And we've seen also that you assisted in giving morphine a little while after this --

LETBY: Yes.

MYERS: -- with Nurse Griffiths. Do you recall during the course of the resuscitation who was -- the stabilisation, however we look at it, who was present in that room. There's you, [Nurse A], Dr Lambie.

Were you joined by anybody else from what you can recall?

LETBY: Um... No.

MYERS: Were there any other doctors present from what you can remember?

LETBY: No, I can't say from memory. Other doctors did come, but I couldn't be sure who.

MYERS: Other doctors came, but you can't recall who?

LETBY: Yes.

MYERS: Do you recall whether [Baby B]'s family were present on this evening?

LETBY: I'm not sure if they were present at the time, but they did come once [Baby B] deteriorated, yes.

MYERS: And who would have dealt with them principally when they came?

LETBY: Usually that would be the designated nurse, so I would assume [Nurse A].

MYERS: Right. If the designated nurse is assisting with the family, what happens with the baby, in this case [Baby B]?

Because she's been stabilised, what happens with her?

LETBY: So the care would still remain with that nurse, but she may need help then from other members of nursing staff that are on the unit to help with the more practical side of the care if she is then supporting the parents.

MYERS: So with that in mind, can we look, please, at the observations at tile 237 and observations that follow the collapse and stabilisation. If we go into that, Mr Murphy, thank you.

Take a look at the chart as a whole and then I want to look at one of the entries in particular. If we look about three or four lines in from the left-hand side, can we see your initials down at the bottom?

LETBY: Yes.

MYERS: And we can see this is the shift on 10 June, going into 10 June. [Nurse A] was the designated nurse for [Baby B], wasn't she?

LETBY: Yes.

MYERS: If we scroll up, we can see the time at which you've done these observations. We have your initials at the bottom. Let's see the row directly above that, so 01.00.

LETBY: Yes.

MYERS: Why would you be doing the observations on [Baby B] at 01.00 if [Nurse A] was the designated nurse?

LETBY: It's not unusual that nursing staff would help each other out with observations. It may have been that [Nurse A] was talking to the parents at that time and has asked me to take the observations for her. It's not unusual.

MYERS: Right. In the normal course of events it would be the designated nurse that deals with the family?

LETBY: Yes, it would, yes.

MYERS: But staff have to continue looking after the baby; is that right?

LETBY: Yes.

MYERS: And is that what you were doing?

LETBY: Yes.

MR MYERS: All right.

My Lord, that might be a suitable point to stop if we may.

MR JUSTICE GOSS: Certainly. Yes, we will do. We'll have a 15-minute break then, members of the jury, and continue at midday, please.

(In the absence of the jury)

MR JUSTICE GOSS: If members of the public could leave the courtroom, please.

(11.46 am)

(A short break)

(12.00 pm)

MR MYERS: My Lord, may I confirm what time your Lordship would wish to rise for lunch?

MR JUSTICE GOSS: I'll leave it entirely to you, Mr Myers.

We have to have at least an hour and 10 minutes at lunch, so if you want it to be just before 1 o'clock, wherever is the most convenient point.

MR MYERS: It's that which I had in mind when I asked.

Thank you.

(In the presence of the jury)

MR MYERS: [Baby C], Ms Letby, which is the third count on the indictment. Can we put up tile 5, please, Mr Murphy, and go behind that?

In fact, if we just have a look at the front sheet, that will probably suffice.

[Baby C], born on 10 June 2015 at 15.31, 30 plus 1 weeks' gestation, born by caesarean section, 800 grams at birth. We can see IUGR, which means intrauterine growth restricted and also reverse end-diastolic flow, which means the umbilical artery flow is reversed to some extent. Are you aware of that?

LETBY: Yes.

MYERS: So that's [Baby C]'s birth details. The events that we have looked at, the key events, before we look at the actual tiles with the nursing notes on, are as follows -- in fact, there's an event on 12 June 2015, not on our sequence of events, but it will come to mind if we look at this, which was a radiograph taken at 12.36, showing marked distension of the stomach and small intestine. That was on 12 June.

Then we come to the event, which is reflected on the charge on the indictment, and that's 13 June at about 23.00 hours when Sophie Ellis gave [Baby C] his first milk feed. And at about

23.15, he collapsed after a projectile vomit. And efforts were made to resuscitate [Baby C], but that resuscitation ceased at 00.45 hours, that CPR ceased then, and sadly [Baby C] died at just before 6 o'clock on the morning of 14 June, that morning.

Let's look at the relevant nursing notes if we may before I ask you questions and let you explain what you can, Ms Letby. The first of those is a note by Sophie Ellis at tile 231. Can we enlarge that? It's the large panel on the left-hand side.

We'll start with the large panel which was a note made at 8.46 on 14 June, so this is several hours after [Baby C] had died. A note by Sophie Ellis who, as we'll see, was the designated nurse.

It says:

"Cares taken over at 20.00. Safety checks completed and fluid requirements calculated. Nursed on Optiflow, 6 litres in 25% oxygen. Observations satisfactory. Heart rate 150 to 157. Respiratory rate 58 to 73, elevated at times as previously been for past few days and temp 37 to 37.1."

Statistics given there:

"Pink and well perfused. Active and alert. On 150ml/kg/day on day 4 due to birth weight of below 1,000 grams. Hourly amount of 5ml."

It sets out the TPN and the lipids:

"Infusing via long line. Site satisfactory. To repeat gas at 02.00 on 14/6/15. Myself asked Reg Katherine Davis to start trophic feeds. She agreed. Started on 0.5ml, two-hourly. First feed of 0.5ml given at 23.00."

That's [Baby C]'s first feed:

"At around 23.15, [Baby C] had an apnoeic episode with prolonged brady and desat. Crash call for neonatal team put out by myself. Resuscitation commenced. Resus drugs given as charted."

We're going to move on in fact, this doesn't involve the vomit, I apologise, this is the brady and desaturation after the first milk feed.

Let's scroll down, please, to the note at 07.38:

"Written in retrospect..."

This is on 16 June, we're just following through from Sophie Ellis:

"Written in retrospect of care from 14/06/15. Had two fleeting bradys, self-correcting, not needing any intervention, shortly before prolonged brady and apnoea requiring resus. Minimal bile aspirates gained. Green in colour. NGT aspirated before first feed. Bile aspirated."

That was an additional note written by Nurse Ellis 2 days after the first note.

Nurse Taylor also provided a note dealing with this and that's at tile 217. Let's go there, please, and we go into that, please. It's on the right-hand side and the second panel, the first with the text in:

"14 June. 07.20."

So again, this is a note written, in this case, a little less than a couple of hours after [Baby C] had died by Melanie Taylor:

"Called to help as baby had brady/desat. When arrived to baby, baby apnoeic, loss of colour. Neopuffed but no able to bag. No chest movement. Guedel airway inserted. Bagged and chest movement gained. Medical team crash bleeped. No heart rate heard. Started chest compressions. No improvement. Continued. Dr Davies arrived. Dr Gibbs called to attend urgently. Intermittent gasping. Continued resus. Intubated by Dr Davies. Positive capnograph for CO2 obtained. Bagged via ETT. Good chest movement and air entry. Continued chest compressions."

It goes on to say:

"Emergency drugs administered as documented. Further resuscitation notes in medical notes. Parents called and present throughout resus. Dr Gibbs present throughout. Still intermittent gasping. Parents wanted [Baby C] to be baptised. Chest compressions and Neopuff continued until baptism finished at parents' wish. Heart sounds then present and gasping."

Then the following note, which we can see, 08.14 that morning, deals with the parental care, [Parents of Baby C] with [Baby C], and sadly [Baby C] dying with them.

Those are the notes that deal with the event, Ms Letby. Again, I make it plain, my error when I referred to vomiting. This is an incident at 23.00 hours on 13 June, when Sophie Ellis gave [Baby C] his first milk feed and he collapsed about 15 minutes later with desaturations and bradycardia.

Before we come to the events of that night, Ms Letby, I'd just like to ask you to clarify something about 12 June. I'd be grateful if Mr Murphy could just put up -- it's not on the tiles because it's not on the sequence of events, but J1996, which is a radiograph from 12 June. We'll just have a quick look at this and J1997 that follows it.

We've had evidence about this from all the prosecution experts in the case. I'm not going to repeat what they say, but that's the radiograph. Then if we scroll down, we can see it's for 12 June at 12.36, a little before midday. If we scroll down to see the commentary at page 1997, please, just to confirm it refers there to:

"Now marked gaseous distension of the stomach and proximal small bowel."

Thank you, Mr Murphy.

That's something which the experts identified as an event. I'd just like to go to your shift pattern, Ms Letby. It's at page 3323. Ladies and gentlemen, you've got the shift pattern in fact in the bundles, but we can put this up on the screen hopefully.

We know that radiograph and whatever it relates to is taken at 12.36 on 12 June. We've got your shift pattern out now. Were you working on 12 June, Ms Letby?

LETBY: No, I wasn't.

MYERS: Or on the unit for that matter?

LETBY: No.

MYERS: When did you come on work in the first -- just for those following, 12 June is in the second row down, isn't it?

LETBY: Yes.

MYERS: It starts with Monday the 8th on the left when you were working nights, the 8th and 9th. That corresponds with events with [Baby A] and [Baby B], doesn't it?

LETBY: Yes.

MYERS: Then you were not working on the 10th, 11th and 12th; is that right?

LETBY: That's right.

MYERS: And you next come in for the evening shift on the 13th?

LETBY: Yes.

MYERS: Right. We can take that down, please, Mr Murphy.

Thank you.

Can we put up, please, tile 16, Mr Murphy. We're going to look at some messages leading up to the dates we're dealing with. Thank you.

So this is 11 June, 14.08, from you to Yvonne Griffiths:

"Hi Yvonne, are you okay for staffing over the next few days? I don't have anything on if you need any extra or need to change my nights."

Tile 17, please. Yvonne Griffiths replies at 17.35 so a few hours later the same day to you:

"Thank you for your kind offer. We are still okay til Saturday night when we only have five staff so we can reassess then."

That's followed at tile 18 and you say:

"Okay. Think I need to throw myself back in on Sat."

Pausing there, what do you mean where it says, "Okay, think I need to throw myself back in on Sat"?

LETBY: That that would be my first shift back to work after [Baby A] and [Baby B].

MYERS: And Saturday is the 13th?

LETBY: That's right.

MYERS: That's actually when you did go in, isn't it?

LETBY: Yes, my shifts changed that week.

MYERS: We touched before on this, but what are you getting at when you are saying, "I need to throw myself back in"?

What do you mean by saying that?

LETBY: That I wanted to get back into the unit and back into looking after babies.

MYERS: Can we go to tile 19, please. Why? Why did you want to get back in? What is it about that that matters?

LETBY: Because that's what I was taught at Liverpool Women's: when you have difficult shifts or babies pass away, the way to sort of overcome that is to go straight back into the environment and carry on.

MYERS: Right. Thank you.

Tile 19, 11 June, 17.45 hours, Yvonne says to you:

"Hopefully it may settle down by then."

Tile 20, please, you say:

"Hope so! But think from a confidence point of view I need to take an ITU baby soon."

Then tile 21, bringing this sequence to an end, Yvonne replies to you 4 minutes later at 17.50:

"Yes, it does knock us a bit when things like that happen, but it's okay to have time out as well. Enjoy the sun."

It says, "It does knock us a bit". Had you been knocked a bit by what had happened?

LETBY: Yes.

MYERS: When you were interviewed about [Baby C], the police asked you about events on 13 June 2015. We're going to look at a few passages from the interview with [Baby C]. Let's reach for the interview file, please. You take it, Ms Letby, you too, ladies and gentlemen, if you would. We're going to go to [Baby C], interview 1, page 1.

It'll say [Initials of Baby C] 1 in red on the top corner.

Just looking down about four lines up, the officer said to you:

"Question: Did you have an involvement with the care of [Baby C]?"

Can you see that?

LETBY: Yes.

MYERS: You said:

"Answer: Just with his resuscitation from what I remember.

"Question: Okay. Can you explain to us what your involvement was then, please?

"Answer: I'm not sure what my exact role was, but I remember him requiring resuscitation."

Pausing there, is that right, at the time of the interview is that as far as your memory went?

LETBY: Yes.

MYERS: The officer says:

"Question: Okay.

"Answer: I remember that he'd not long had his first feed by one of the nurses and it wasn't long after that that he deteriorated, so I am not sure what my role was with him in that resuscitation. I think I did chest compressions. And I remember we waited for the vicar to call and we carried on resuscitation efforts until the vicar arrived and then we withdrew care and [Baby C] lived for several hours after that around -- in the parents' accommodation.

"Question: Okay. Are you aware of why he required resuscitation?

"Answer: No, I don't recall the events leading up to that. I just know that he had resuscitation."

Is that right, did you have any recollection of any events leading up to the resuscitation?

LETBY: No.

MYERS: And you are asked:

"Question: So you're not aware of what the circumstances of his collapse were?

"Answer: No.

"Question: Had you had contact with him to that point?

"Answer: Not that I recall, no."

How easy was it to recall detail when you were being interviewed in July 2018 of what happened on this night leading up to this collapse?

LETBY: It was very difficult. I don't really have any memory until the resuscitation with it being a significant event. Prior to that, it was just a normal shift.

MYERS: Had you had any involvement with [Baby C]?

LETBY: No.

MYERS: Or any reason for it to stand out in any particular way?

LETBY: No.

MYERS: How easy is it just to go back and think up details relating to a baby whose name is given to you, maybe years after the event?

LETBY: It's very difficult. I've looked after hundreds of babies.

MYERS: But the events of this night, did they stand out though?

LETBY: Yes.

MYERS: Did you know who the police were talking about when they asked you about [Baby C]?

LETBY: I did, yes.

MYERS: Let's go through events as you recall them, Ms Letby.

Can we look at the layout, please, for the start of this shift, which is at tile 138. We'll be going back to the interview in a little bit.

Scroll down a little bit more so we can see who was on, please.

We don't have a diagram for this, but we have got the tables. This shows the staff present: shift leader [Nurse B]; designated nurse, Sophie Ellis.

If we scroll down we can see who you were designated for, Ms Letby. You were for baby JE and baby PE; can you see that?

LETBY: Yes.

MYERS: Do you recall which nursery you were in or nurseries?

LETBY: Nursery 3.

MYERS: And do you recall which nursery [Baby C] was in?

LETBY: Nursery 1.

MYERS: When it comes to recalling what took place that night, how much are you able to recall on your own recollections and how much on other matters given to you to assist with recollections?

LETBY: I have very little independent memory.

MYERS: Right. Little independent memory?

LETBY: Yes.

MYERS: We're going to have a look at the neonatal review then to see the movements leading up to this time. It can be put on the screens. Certainly keep the interview bundle there, ladies and gentlemen. By all means follow it if you wish to in paper, but I'm going to ask Mr Murphy if he could put up page 3 of 9 of the neonatal review for [Baby C].

We can see the event for [Baby C] at 23.15 is right in the centre at line 71; do you see that?

LETBY: Yes.

MYERS: If we go up to the top half of the chart and enlarge that so we can see what you're recorded as doing in the period leading up to that. Can you see line 55?

LETBY: Yes.

MYERS: What does that show you doing?

LETBY: So that's me carrying out observations on the baby JE.

MYERS: And observations -- and what about line 56?

LETBY: I also did the fluid balance chart at that time as well.

MYERS: That's at 23.00 hours; yes?

LETBY: Yes, so that was an observation and fluid balance and a feed.

MYERS: Which nursery would you be in for that?

LETBY: Nursery 3.

MYERS: Line 65, also in blue, what's happening then also at 23.00?

LETBY: So around that time, I've also done observations on my other baby, PE.

MYERS: Which nursery would that be in?

LETBY: I believe that was nursery 3 as well.

MYERS: Yes. Was anybody else working in nursery 3 with you as far as you can recall now with everything you have learnt?

LETBY: [Nurse B] was around and about quite often, yes, but I don't know if she was actually working in the nursery.

MYERS: All right. Looking after JE and PE and carrying out the type of tasks that we can see at 23.00, what sort of timescale does that require to do those tasks across those babies approximately?

LETBY: The observation chart for PE would probably have taken a matter of minutes. That would have been readings from a monitor I assume. With JE it would have been longer because I believe I gave a milk feed at that time as well.

MYERS: Right. From what you can see would you have been anywhere other than nursery 3 in or around 23.00?

LETBY: No.

MYERS: Before we move forward, I'd like to see what led up to that. Could we go to page 2 of 9. We know [Baby C], the way these tables are dealt with, is in the yellow, the yellow bars are [Baby C], that's the baby we're looking at. And you, Ms Letby, are in the blue.

LETBY: Yes.

MYERS: You have said to us you don't recall having had anything to do with [Baby C] that evening?

LETBY: No.

MYERS: What were you occupied with that evening?

LETBY: The babies I was allocated to look after in nursery 3.

MYERS: We know the collapse with [Baby C] is round about 23.15.

LETBY: Yes.

MYERS: We know he had a feed at 23.00.

LETBY: Yes.

MYERS: And we've seen what you were doing at 23.00.

LETBY: Yes.

MYERS: If we look at this, starting at line 28, we're going to just look down to the end of the table, lines 28, 29 and 30, 22.00, involving baby JE. What would you have been doing at that time?

LETBY: So I've also made an entry on my nursing notes to reflect some observations that I've made of JE at that time.

MYERS: And which nursery is that in?

LETBY: Nursery 3.

MYERS: Move down, please, to line 37.

LETBY: There I've taken fluid readings for my other baby, PE.

MYERS: Lines 38 and 39; can you help us with that?

LETBY: Yes, they're medications that I have co-signed with [Nurse B] for a baby, HM. I'm not sure where that baby's located.

MYERS: I'll tell you: HM is allocated to Nicola Dennison, as it happens, on the schedule we have.

Carrying on: EB at 40 and 41. What's going on there?

LETBY: So the same again, myself and [Nurse B] have given medication for this baby.

MYERS: And lines 45 and 46, what are you doing there?

LETBY: So that's another set of drugs for another baby, a different baby again.

MYERS: We've seen what follows with your babies in nursery 3 at 23.00. So again, assisted by that, did you have any involvement with [Baby C] before the time of the collapse?

LETBY: No.

MYERS: And so far as you can remember and from what you've seen, where were you located?

LETBY: In nursery 3.

MYERS: What was the first you were aware of any problem relating to [Baby C]?

LETBY: Once he'd collapsed.

MYERS: If you just stay focused with this, Ms Letby. How easily do things distract you around you if anything happens?

LETBY: I'm very easily distracted.

MYERS: Have you always been like that?

LETBY: No.

MYERS: You do your best to concentrate on this.

Right. What I was asking, before that, was: what was the first you were aware of any issue with [Baby C]?

LETBY: So when he collapsed.

MR JUSTICE GOSS: You did actually say after he had collapsed. It may be a distinction without a difference. I think she had originally said after he'd collapsed and then said when he collapsed. I just wonder if --

MR MYERS: That's why I wanted to go back over it. There's a certain amount of --

MR JUSTICE GOSS: Yes.

MR MYERS: -- activity. So I'm trying to get focus back on this.

MR JUSTICE GOSS: Absolutely, let's get back and focus.

Perhaps start again.

MR MYERS: I will.

I've been asking about where you were working, Ms Letby.

LETBY: Yes.

MYERS: And you described to us what you were doing in nursery 3.

LETBY: Yes.

MYERS: What was the first you were aware of, of any event concerning [Baby C]?

LETBY: At the time of his collapse.

MYERS: At the time of. Can you give us a bit more detail than that? When your recollection kicks in with detail, what is it?

LETBY: So he was receiving support at that time and I was called to help.

MYERS: Right. Before you were called to help, were you aware of anything taking place with [Baby C]?

LETBY: No.

MYERS: Doing the best you can to recall this, where were you before you were called to help?

LETBY: I wasn't in nursery 1.

MYERS: Right. Do you recall who called you to help?

LETBY: I believe it was Sophie Ellis.

MYERS: Once you'd been called to help, what happened next?

LETBY: I don't have a lot of independent memory, but I believe I asked Sophie to go and put out a crash call and Melanie Taylor was with [Baby C].

MYERS: Let's just pause a moment, you've talked about Melanie Taylor now. When were you first aware of Melanie Taylor?

LETBY: Melanie Taylor was in the nursery when I arrived.

MYERS: Right. When you arrived?

LETBY: Yes.

MYERS: So you had been called to help?

LETBY: Yes.

MYERS: Where did you go after you were called to help?

LETBY: Into nursery 1, to [Baby C].

MYERS: Once you went there, do you recall what you could see?

LETBY: Mel was with [Baby C]. I just remember that he was apnoeic and needed respiratory support, so I asked Sophie to put out a crash call.

MYERS: Right. Do you recall anybody else being present at that point?

LETBY: I believe [Nurse B] was there as well.

MYERS: Do you believe that from your own recollection or do you believe that from things you've heard since then?

LETBY: Um, no, I do have some recollection of that.

MYERS: That she was there?

LETBY: Yes.

MYERS: Who was it that put out the crash call?

LETBY: Sophie Ellis.

MYERS: Did you offer any assistance or play a part in what happened after that?

LETBY: Yes, so from that point, full resuscitation commenced and I know I did take part in chest compressions at certain times.

MYERS: Do you remember the particulars of what took place?

LETBY: No.

MYERS: I just want to go through some of the questions you were asked about this when you were interviewed by the police. I would like us to go to the [Baby C] interviews, if you would, please, ladies and gentlemen, behind tab 2. So [Initials of Baby C] and this time it's 2.

It'll have [Initials of Baby C] 11 in red at the top right-hand corner if we're at the right place.

There's a passage I want to remind us of and then a couple of questions about that. Can you see on the left-hand side of the page it says 00.29.10?

LETBY: Yes.

MYERS: And the officer is talking about Nurse Sophie Ellis, who's been spoken to. This is the officer reporting this:

"Question: She was out of the room at the nurses' station when [Baby C]'s alarm sounded. She says that you had your own designated baby who was in nursery 3. Do you agree with that?

"Answer: No, as I've said before I don't remember who I was looking after on that shift, I would have to look and check."

Just pausing there, why do you know it was nursery 3 now?

LETBY: Because when I was informed of the babies' initials of that evening shift, I could recall that baby.

MYERS: You are asked:

"Question: She goes on to say that when she went into nursery 1 in response to alarms she says that you were standing next to [Baby C]'s cot as you entered. Do you agree with that?

"Answer: I don't specifically remember when I entered the room or why I entered the room."

So as matters stood do you agree that you were standing next to the cot?

LETBY: No.

MYERS: Do you have any recollection that you were standing next to the cot?

LETBY: No.

MYERS: Do you recall alarms going off in particular?

LETBY: I recall alarms going, yes.

MYERS: You do recall alarms?

LETBY: Yes.

MYERS: You told the police:

"Answer: I don't specifically remember when I entered the room and why.

"Question: Nurse Sophie Ellis says that you said, 'He's just dropped his HR and sats', or something similar. Do you recall saying that to Sophie Ellis?"

And you said no.

Now looking back at this, do you recall saying anything like that to Sophie Ellis?

LETBY: No.

MYERS: Over the page. Just this page that I want to deal with, page 112:

"Question: You were placed into nursery 3 during his shift and had your own designated baby to look after; do you agree with this?"

"Answer: Not from memory, I'd have to check if that's documented as being right then, yes, it would have. I don't remember.

"Question: Okay, Lucy, have you got any explanation as to why you were already in nursery 1 when [Baby C]'s alarms were sounding?"

Pausing there, did you have any recollection of being in nursery 1 prior to the alarms sounding?

LETBY: No.

MYERS: Or of what Sophie Ellis had said to the police?

LETBY: No.

MYERS: You say:

"Answer: I don't recall from memory. I may have been in there doing the checks that we do in the ITU room, I may have been getting a drug out of the cupboard, I might have been using the computer, I might have heard his alarms, I don't recall."

Why were you suggesting all those things if you didn't recall where you were?

LETBY: Because based on the interview of Sophie Ellis, she had placed me in the nursery.

MYERS: Do you mean based on the statement of Sophie Ellis?

LETBY: Yes, sorry, yes.

MYERS: What the police were saying to you?

LETBY: Yes.

MYERS: The police say:

"Question: What checks are they, Lucy?

"Answer: There's the resus trolley that's in the room and the ITU spaces are all checked each evening.

"Question: Would you do all that whilst caring for other babies then?

"Answer: Yes."

LETBY: it happens are those checks a nurse might do whilst caring for other babies?

LETBY: Yes, they are, yes.

MYERS: But had you actually been in there, in that room, when [Baby C] desaturated?

LETBY: No.

MYERS: Question:

"Had you been treating [Baby C] at this time, Lucy?"

You said:

"Answer: Not that I remember."

Pausing there, did you have anything like the neonatal review that we have now to refer back to when you were being interviewed?

LETBY: No.

MYERS: Question:

"Have you got any explanation as to why you were stood at [Baby C]'s cot side as Sophie Ellis has told us?

"Answer: Not as I -- no. As I say, not from memory. I don't recall why specifically I was there."

Pausing there, had you actually remembered that you were there?

LETBY: No.

MYERS: Why were you talking or beginning to talk about being there then?

LETBY: Because from the statement of Sophie Ellis, that's what had been put to me, that I was there.

MYERS: Right.

We know that -- if we actually read just to the end of this section. The comments -- she talks about the comments that Sophie Ellis said you'd made:

"Question: Got any explanation as to why you made that comment to Sophie?

"Answer: That's what I must have witnessed, alarming, on the alarm, and that's when she's come."

Did you actually have any recollection of being there with the alarm sounding and saying anything to Sophie?

LETBY: No.

MYERS: Sol why were you talking about saying things to Sophie?

LETBY: Because that was the suggestion, that Sophie had said I was there and I was thinking of reasons, if that was correct, why I would have been there.

MYERS: And the officer says:

"Question: Okay, you've got no explanation as to why you're in nursery 1?

"Answer: No. As I said I don't recall."

LETBY: Mm.

MYERS: We know that also in 2018, [Nurse B] had made a statement to the police. That's in evidence, we've heard that. She was asked about that when she gave evidence. Did the police at any point, when faced with you saying you didn't recall things, tell you about what [Nurse B] had said in her statement?

LETBY: No.

MYERS: Was there any reference to what she'd said, that she'd been dealing with JE and Lucy when some time, between 10 and 11, you were called to assist with [Baby C] or rather she, [Nurse B], was called to assist with [Baby C]?

LETBY: No.

MYERS: Any reference to that to assist with your memory?

LETBY: At the police interview?

MYERS: Yes.

LETBY: No.

MYERS: And again with regard to the statement from [Nurse B] dated 2018, was there any reference to you to assist with memory that she had said that when she entered nursery 1:

"Sophie and Melanie were Neopuffing [Baby C] in an attempt to assist with breathing"?

LETBY: No.

MYERS: Did they assist you with that?

LETBY: No.

MYERS: And also from that same statement, raised with her in evidence in this trial, when describing the attendance of the doctors to intubate [Baby C], she said:

"I also think Lucy was in the room by now as well."

Was your memory assisted with that?

LETBY: No.

MYERS: Whatever you said in response to what was identified from Sophie Ellis, did you have an independent recollection of what took place that night?

LETBY: I had some recollection.

MYERS: Starting from where?

LETBY: So I believed that I had been called to help.

MYERS: Right. Had you had any involvement with [Baby C] before that?

LETBY: No.

MYERS: Or any recollection of involvement before that?

LETBY: No.

MYERS: Did you accept as accurate what the police were telling you with regard to Sophie Ellis?

LETBY: Yes.

MYERS: And by that I mean what she'd said in her statement, you accepted that from the police?

LETBY: Yes, so from what they told me, I assumed that to be true.

MYERS: Right. Also referred to in the course of the interviews, I want to ask you about this, were messages that you'd sent whilst on duty that evening, earlier that evening --

LETBY: Yes.

MYERS: -- messages between you and Jennifer Jones-Key. So could we look, please, at tile 147? Thank you.

This is a message from you to Jennifer Jones-Key at 21.48 hours on 13 June. So you were at work?

LETBY: Yes.

MYERS: "Keep thinking about mum, feel like I need to be in 1 to overcome it but [Nurse B] said no."

Is that what had happened, you'd asked [Nurse B] and she'd said no?

LETBY: I can't remember specifically now, but it reads that way, yes.

MYERS: And if [Nurse B] said no, what did you then get on with?

LETBY: Taking care for the babies that I was allocated to look after.

MYERS: Tile 148, please. Jennifer Jones-Key says:

"I agree with her. Don't think it will help. You need a break from full-on ITU. You have to let go or it will eat you up. I know not easy and will take time."

We know -- do you recall this series of messages that took place at this time?

LETBY: I recall the content of the conversation, not the full details, but yes.

MYERS: We know the messages run over a couple of hours. I'm not going to all of them, we've seen where they start, but I want to go towards where they end because that's something which the police pointed out to you in the interview. So if we go to tile 174, please.

This is 23.01. You're sending a message to Jennifer Jones-Key saying:

"Women's can be awful but I learnt hard way that you have to speak up to get support. I lost a baby one day and few hours later was given another dying baby just born in the same cot space. Girls there said it was important to overcome the image. It was awful but by end of day I realised they were right. It's just different here."

And tile 175, please:

"Anyway, forget it [this is a minute later]. I can only talk about it properly with those who knew him and Mel not interested so I'll overcome it myself. You get some sleep."

And Jennifer Jones-Key replied to you at tile 176:

"That's a bit mean, isn't it? Don't have to know him to understand. We've all been there. Yep, off to bed now."

Pausing there, we have seen that you were involved with various cares and charts at 23.00, it says. Would that be going on at the same time as you're texting?

LETBY: No.

MYERS: So can you help us with whether or not you'd have done those observations before or after the texts?

LETBY: I couldn't say. It'd be around that time, but it's not a definitive time on the chart.

MYERS: Can we go to tile 178, which is the next tile in this sequence. Jennifer Jones-Key had said it was a bit mean, this is at 23.09, and you say:

"I don't mean it like that, just that only those who saw him know what image I have in my head."

And you say at tile 179:

"Forget it, I'm obviously making more of it than I should."

And then tile 180:

"Sleep well."

In interview, having gone through the evidence of Sophie Ellis, it was then identified those texts took place and it was

suggested to you that you then went on to attack [Baby C]. Do you recall that being put to you?

LETBY: Yes.

MYERS: Is there anything about those texts which have the effect of making you want to attack any child?

LETBY: Not at all, no.

MYERS: You have been talking about how you wanted to -- how you learnt at Liverpool Women's how to deal with the loss of a baby, had you?

LETBY: Yes.

MYERS: Was that something you wanted to happen? Take a drink.

LETBY: Sorry, could you repeat that?

MYERS: The loss of a baby, is that something you wanted to happen?

LETBY: Not at all, no.

MYERS: Why were you talking about the support you needed for it the way to deal with it?

LETBY: What do you mean, sorry?

MYERS: Why were you talking about Jennifer about how best to deal with the loss of a baby and what should happen?

Why was that coming up? What was on your mind, Ms Letby, that made you want to talk about where you should be working?

LETBY: What had happened with [Baby A].

MYERS: Yes. Was that something you wanted to happen again?

LETBY: No.

MYERS: Do those messages have anything to do with what happened with [Baby C]?

LETBY: No, not at all.

MYERS: Do you recall whether you had any contact with [Baby C]'s family in the immediate aftermath of what took place?

LETBY: I know the family were present whilst resuscitation was ongoing and I was part of the resuscitation process, so yes, they were there.

MYERS: And did you have any contact with them from what you can remember?

LETBY: I can't recall specific contact, no.

MYERS: And did you have any contact with [Baby C] in particular after he died? Rather, let me say, after CPR had been stopped.

LETBY: I can't recall.

MYERS: All right. So is that to say you might have done, you can't remember, or you didn't?

LETBY: I may have done, but I couldn't recall specifically.

MYERS: Do you recall whether you assisted with any of the steps that are taken to help families come to terms, as much as they can, with the enormity of what's just happened?

LETBY: Not from memory.

MYERS: We know that you did a Facebook search for [Parents of Baby C] on 14 June at 15.52. So that's later on the day that [Baby C] had died. Why did you do that?

LETBY: The family were very much on my mind. The first time I had met the parents was during the resuscitation of their son.

MYERS: And why were they on your mind, if that isn't too obvious a question?

LETBY: Because when you go home from work, you don't forget about the babies that you've cared for and what's happened.

MYERS: We know that in their case and other cases there are times you do searches on Facebook in the months that follow.

LETBY: Yes.

MYERS: Why is that?

LETBY: Because there are times when they do enter my mind.

MYERS: How do you feel about what they have been through?

LETBY: What the parents have been through?

MYERS: Yes.

LETBY: It's unimaginable. I can't...

MYERS: All right. Pause a moment.

I want to ask you some questions next about [Baby D]. We can commence --

MR JUSTICE GOSS: It's up to you, Mr Myers. If you want to start or if you want to have the break now. We would then resume at 2 o'clock. It's up to you. As you recall, we have to have an hour and 10 minutes.

MR MYERS: Maybe given what we're dealing with and where we've reached in the evidence at this point, it might be better to have the break and the extra 10 minutes now if that's convenient. It's still the break your Lordship wished. This is a more natural place to stop.

MR JUSTICE GOSS: It's not the length of break I wish, it's that circumstances dictate it has to be.

MR MYERS: Thank you.

MR JUSTICE GOSS: If we break off now, could you be ready to continue at 2 o'clock, members of the jury? And remember your obligations as jurors.

(In the absence of the jury)

2 o'clock for everyone to come back.

(12.49 pm)

(The short adjournment)

(1.58 pm)

(In the presence of the jury)

MR MYERS: Ms Letby, I'm going to turn to count 4 on the indictment, which is the case of [Baby D]. Can we put up tile 7, please, Mr Murphy. Can we look at the tile just directly behind that, please? That will probably suffice if we leave it there.

We see [Baby D], Ms Letby, born on 20 June 2015, 16.01, 37 weeks and 1 day, 3.130 kilograms, caesarean section. We may recall, I'll just remind us now, we had it established in agreed fact 22, that there had been a spontaneous rupture of the membranes 60 hours before birth in the case of [Baby D].

Could we turn to tile 19, please, for her condition very soon after birth? If we go into that, please.

This is a note by Dr Rylance and it's just the central part we are going to look at. It's got "BM 4.2"

underlined in the margin. Then it says this:

"First baby. Uncomplicated pregnancy. No known GBS. Well in labour. Apgars 8 and 9. Initial poor colour. 12 mins age, in dad's arms, lost colour, floppy. Five rescue breaths and 2 mins IPPV. Reviewed by SHO. On arrival, good respiratory effort."

Then it says:

"Started grunting in theatre. Reviewed at 1.5 hours by SHO. Grunting but otherwise obs okay and examined normally. Bleeped again as midwife not happy with her colour and lack of response to vit K. Poor feeding. Brought to unit for screen. On arrival to NNU, looked dusky, poor respiratory, initial sats 48%. Poor respiratory effort. Around 19.30 [then it's got] transferred into incubator."

And the treatment after that. She was then stabilised after that. But that's just the period immediately after the birth and the reference to what happened when she was 12 minutes of age, something we heard about in the evidence.

So that is the situation with [Baby D], Ms Letby, around the time of birth. The actual events that we look at, we count four on this indictment, and there were three elements to it -- I'll just remind us of this and then we'll come to the note shortly in evidence.

We're looking at the early hours of the morning of 22 June 2015. At 01.30, there is a collapse with [Baby D] and discolouration noted. And there was a recovery from that.

LETBY: hour and a half later, 03.00, there was a collapse and some discolouration, noted to be less than the first time, and there was recovery from that.

Then at 03.45, that same morning, 22 June, there was a collapse, no discolouration noted on that occasion, but sadly [Baby D] did not recover and she died at about 04.25 that morning. So that's just to outline the case with [Baby D].

I just want to start with what your memory was of this when you were first questioned about it, Ms Letby, and therefore could we just look inside the interview bundle, please, ladies and

gentlemen, to the [Baby D] interview, behind the first tab for [Baby D]. We've got [Initials of Baby D] 1 in the top right-hand corner. We can see just after the summary in blue, that's the first [Initials of Baby D] tab, at the bottom of the page, it says:

"Question: Do you remember [Baby D] at all?"

Can you see that?

LETBY: Yes.

MYERS: Over the page, you say, "No".

You confirm that you'd had a copy of the relevant notes and charts. Your attention was drawn to the signature on one of them, the officer says:

"Question: Right, you're the first signature on those. From that do you presume that you then administered --"

There was a prescription chart. You said yes.

There's a bit of summary, then the officers says:

"Question: Is there anything you can tell us about what happened to [Baby D]?"

And you said:

"Answer: I don't really remember [Baby D]."

That was the position at the time of that police interview.

LETBY: Yes.

MYERS: Is that the case, that but for charts and the like that had been identified, it's not something you remember?

LETBY: No.

MYERS: We'll go straight to the shift we're looking at, which is the evening shift from the 21st into 22 June, and let's look at the layout. It's at tile 168, please, Mr Murphy. Thank you. Go behind that.

We can see, looking at that, particularly with the nursing staff, the shift leader was [Nurse C].

Caroline Oakley was the designated nurse and you were also on duty, with Kathryn Percival-Calderbank, as she is now, and

Elizabeth Marshall. If we scroll down, we can see who is where so far as the layout is set up.

We can see that [Baby D] was in nursery 1 with Caroline Oakley, and you were also in nursery 1 with babies MRE and JE.

What I would like to ask you about first is something in the evening of 21 June and it relates to evidence from [Baby D]'s mother, [Mother of Baby D].

[Mother of Baby D] gives a description of seeing a nurse that she says was you in the neonatal unit at 7 pm on 21 June. Do you remember that in [Mother of Baby D]'s evidence?

LETBY: I do, yes.

MYERS: She says that she also saw you at some point during the collapse of [Baby D] on the morning of the 22nd, but she said in her evidence that she'd been in the unit at 7 pm on 21 June, that someone she identified as you -- and these are the words that was used:

"She had seen hovering around [Baby D] not doing much and had a clipboard with notes on it and said [Baby D] was fine."

And she said she was clear this was at 7 pm or 19.00 hours because she had looked at the clock and she recognised the person she says is you from images on the media. Do you remember that in the evidence?

LETBY: Yes.

MYERS: So far as you can recall, did you see [Mother of Baby D] and speak to her at that time on 21 June?

LETBY: No.

MYERS: Would you have been, so far as you can recall, on the unit at 19.00 hours on 21 June?

LETBY: No.

MYERS: We see that you're on the evening shift. What time would you expect to come in for that shift?

LETBY: So the usual behaviour for me would be any time after 7 o'clock, I would enter the unit and get ready for the shift, but I wouldn't be in the clinical nurseries necessarily at that time.

MYERS: Right. Could we look at tile 163, please? Just go behind that. This is swipe data, the first time your name appears in this period, and it's for 19.26 on 21 June. We can see it's "maternity neonatal entrance doors in".

Can you help us with what that might correspond with, that swipe data at 19.26 that evening?

LETBY: So that would reflect me swiping on to the neonatal unit at that time through the main entrance doors.

MYERS: What would you be going in for at that time?

LETBY: To prepare for my shift.

MYERS: Could we go to tile 159, please? We're just going to have a look at that. This is a text message from your phone to someone called Charlotte James at 19.15 that evening, so 11 minutes before the swipe data that we saw. At that time you send a message saying:

"I'm just about to leave for a night shift so no problem. Hope all is okay."

It might be clear, but what are you talking about when you say that?

LETBY: So this was in reference to me at this time, I was living in the staff accommodation on site at the hospital, Charlotte James was somebody that I lived with at that point, and she'd asked me something about that evening and I'm telling her I'm just about to leave for a night shift.

MYERS: So where would you be when you sent that so far as you can tell?

LETBY: At Ash House.

MYERS: Yes. Would you have been on the unit?

LETBY: No.

MYERS: Would you have been on the unit at 19.00?

LETBY: No.

MYERS: Where are you heading to at 19.15?

LETBY: I'm about to leave my house and go over to the unit.

MYERS: In the ordinary course of events, who would be responsible for [Baby G] before your shift commenced?

LETBY: You said [Baby G].

MYERS: Sorry, for [Baby D], I apologise. Who would be responsible for [Baby D] before your shift commenced?

LETBY: Whoever the designated nurse was on the day shift.

MYERS: Yes. Can we go to tile 96, please. Have a look at that, please. This is from that morning, 07.30 to 08.00, handover. Who's the designated nurse?

LETBY: Kate Bissell.

MYERS: And for how long would Kate Bissell, in the ordinary course of events, be looking after [Baby D]?

LETBY: Until 20.00.

MYERS: Can we put up tile 147, please, which is some notes by Kate Bissell. Enlarge those, please. We don't need to read all the content, but when we see at the top there, 21.06, "At 17.03 KB", what's happening at 17.03?

LETBY: Kate Bissell is logged into the computer system and is documenting on [Baby D]'s care notes.

MYERS: Just scroll down to the entry below that, please.

21/06/15 at 19.45 now. What's happening there?

LETBY: So again, Nurse Bissell has logged on to the system at 19.45 and is entering notes into [Baby D]'s system, care system.

MYERS: Can we look at tile 27, please, which is one of the intensive care charts. Actually go into the chart, please, Mr Murphy. Thank you.

This is 21 June. [Mother of Baby D] was talking about a time at 19.00 on 21 June. If we look across to the right-hand side, can you see the timing for 18.30, the farmost column on the right?

LETBY: Yes.

MYERS: That's the observations for [Baby D] at 18.30 on 21 June.

Can we scroll down to see whose initials are there, please? We might need to enlarge it down at the bottom right.

The initials that are there at the bottom right, are they your initials?

LETBY: They're not, no.

MYERS: Do you recognise whose initials that would be?

LETBY: They look like Kate Bissell's.

MYERS: All right. Could we go to the chart that follows this at tile 164, please. Just go into that, please. We can see on the left-hand side -- can you see there's an entry by someone at 19.30 --

LETBY: Yes.

MYERS: -- for [Baby D]? So we've seen Kate Bissell at 18.30. Can you see -- let's see which initials are there for 19.30. Scroll down, please. Lower left.

LETBY: Again, I would say that's Kate Bissell.

MYERS: Are they your initials?

LETBY: They're not, no.

MYERS: Right. So we've seen Kate Bissell's initials on charts at 18.30, 19.30, nursing notes by her, and that she was the designated nurse for [Baby D]. To the best of your recollection, were you even on the unit, in fact, from 19.00 -- from what you have seen, were you on the unit at 19.00 that evening?

LETBY: No.

MYERS: All right. Thank you. We can take that down, please, Mr Murphy.

I want to move then to what happens later in the shift when we come to events with [Baby D] and the deteriorations that took place. The starting point -- let's have a look at Caroline Oakley's note, which sets out what happened, and then we can go through what you have to say, Ms Letby. Can we put up tile 228, please? Thank you.

We'll go to the actual note. There's quite a lot of detail here, but let's have a look at it because it's a while since we've seen it. It's the right-hand side going up to the top, thank you, Mr Murphy.

This is 22 June 2015 at 04.46. CO, is that in all probability Caroline Oakley?

LETBY: Yes.

MYERS: Yes, it is, right. This is 20 minutes after [Baby D] has died. It's written in retrospect for care given from 20.00 to present:

"Handover taken. Equipment, alarm limits and IV fluids checked and satisfactory. UVC in situ for sampling only as low lying. X2 peripheral lines in situ right and left hands. Lower limbs dusky and feet bruised. Doctors aware. Feet cool to slightly warm. Observations satisfactory. [Baby D] nursed on N CPAP in air. Antibiotics given as prescribed. Reviewed by Dr Brunton. Commence feeds and increase as tolerated. EBM and consented to donor EBM. Bloods/gas taken from UVC by Dr Brunton. Gases at 23.52 and 01.14 satisfactory. Bilirubin below treatment line. Repeat CRP 6.

"01.30. Called to nursery by Staff Nurse Percival-Ward and Staff Nurse Letby. [Baby D] had desaturated to 70s, required oral suction as was bubbly and had lost colour. Discolouration to skin observed, trunks, legs, arms, chin. Dr Brunton called to review. Saturations 100% and O2 weaned to air. Observations satisfactory. Dr Newby called in to review. Fluids increased to 90ml/kg/day. Cefotaxime commenced. Benzylpenicillin increased to TDS and Na infusion commenced as Na 127."

Can you help us with what Na infusion means?

LETBY: It's a sodium infusion. Na is sodium.

MYERS: "Saline bolus as prescribed. Discolourations resolved.

AXR [abdominal X-ray] taken and satisfactory. Continue supportive treatment, maintain UVC. Decision to speak to parents later as [Baby D] stable and doctors required on paed. Repeat gas good.

"03.00. [Baby D] crying and desaturated again to 70s. Commenced on 100% O2 via CPAP and picked up well but skin discoloured again, but less than previously. Dr Brunton called to review. Taken off N CPAP. Further fluid bolus and gas 1 hour. Cares attended to. Passed urine +++ and passed meconium. Observations satisfactory. [Baby D] settled and handling well.

"03.45. Monitor alarming. [Baby D] desaturated and then became apnoeic. Called Staff Nurse Letby to help. Stimulation given with no effect. Bagging via Neopuff at 3.52. SHO on unit and called to help. Dr Brunton crash called and resus commenced as per medical notes."

We can see beneath that, very sadly, the entry at 06.08, we can see it on the screen:

"CPR stopped at 04.21 and time of death 04.25."

Scroll back up a little bit, please. We see at 01.30:

"Called to nursery by Staff Nurse Percival-Ward [who we know is Percival-Calderbank now] and Staff Nurse Letby."

Do you have any particular recollection of that taking place?

LETBY: No, I don't, no.

MYERS: Do you know what you had been doing leading up to that?

LETBY: No.

MYERS: Do you know which nursery you would have been in?

LETBY: No, I have no memory of this event.

MYERS: We saw that you had two babies you were designated to in nursery 1.

LETBY: Yes.

MYERS: In the ordinary course of events, which nursery would you expect to be in if you had babies in nursery 1?

LETBY: So predominantly in nursery 1.

MYERS: Right. Let's have a look in the neonatal review just to see if that assists with what took place around about these times. We're going to look in the neonatal review -- and we can put it on the screens, ladies and gentlemen. Mr Murphy is able to assist with that -- for [Baby D]. Can we go to page 3, please. Thank you.

Is that page 4 there? Thank you very much.

If we perhaps just start with midnight, which is line 80. Can you help us with what you were doing?

LETBY: I can't read that.

MYERS: I apologise. Let's enlarge line 80 a little bit more.

It's quite small. Thank you.

I'll read it into the record, what we see there:

" 22 June, midnight, 00.00, MRE."

And there's two entries for 00.00, the IC observations and neonatal fluid balance, and your name against those.

LETBY: Yes.

MYERS: So if MRE is in nursery 1, is that where you'd be --

LETBY: Yes.

MYERS: -- doing those? If we just pull out now and look at the chart, the blues and yellows. We can see blue and yellow are separate, Ms Letby, until we get to line 116, so perhaps let's go there and enlarge that.

Line 116, 01.25, there's an infusion that commences at that time, and if we look across we'll see that's you and Caroline Oakley with [Baby D].

LETBY: Yes.

MYERS: There we are. Thank you.

Do you recall that taking place?

LETBY: No, I don't, no.

MYERS: And do you have any recollection of how it was, at 01.30, Caroline Oakley was called to the nursery by you and Nurse Percival-Calderbank?

LETBY: No.

MYERS: We've seen nurses assisting one another with medications and with help on the unit.

LETBY: Yes.

MYERS: Is that something which would happen commonly?

LETBY: Yes, particularly if two nurses are both working in nursery 1 in the same nursery.

MYERS: But in addition to assisting Nurse Oakley with this, what other types of tasks would you have been doing in nursery 1 on this evening?

LETBY: So I would have been caring for the babies that I was allocated to and helping anybody else on the unit that needed

any assistance with their medications. There's also jobs to do at night that we have to do, equipment checks and various things.

MYERS: If we go back one page, please, to page 2, an example of what you were describing. Lines 64 and 65 on page 2 down at the bottom. This is 23.43, prescriptions for the baby EB. If we just see who's involved with that.

That's Kathryn Percival-Ward and you?

LETBY: Yes.

MYERS: You said you were helping other nurses where necessary with what they were doing?

LETBY: Yes.

MYERS: Is that quite commonplace?

LETBY: Yes, you have to have two nurses so inevitably there will be times when we have to work together.

MYERS: And you were in nursery 1?

LETBY: Yes.

MYERS: So you agree you assisted Caroline Oakley at 01.25 with an infusion for [Baby D]?

LETBY: Yes.

MYERS: Do you recall any detail of [Baby D] desaturating and what happened?

LETBY: No.

MYERS: Do you recall any of the detail of what happened with the collapse about an hour and a half later at 03.00?

LETBY: No.

MYERS: We're still with the neonatal review, so let's have a look at page 4, just to see events so far as that assists leading up to that. If we go, please -- we're looking at lines 155 to 162. Pausing first, as we look at that we can see you're involved with various tasks in between the time of the two collapses; is that right?

LETBY: Yes.

MYERS: Can we look at lines, first of all, 142 to 145, in fact?

It's a little difficult maybe with the pale blue, but can you see that or is it difficult to see, Ms Letby?

Say if it is difficult.

LETBY: I can't see the initials but I can see the rest of the information.

MYERS: Well, we've got it there and I'll read it so you can follow. JE is one of the babies you were designated to look after.

LETBY: Yes.

MYERS: What we're looking at here is an entry or entries at 02.18 for JE, prescriptions. And who's dealing with them, can you see that?

LETBY: Yes, myself and Caroline Oakley.

MYERS: Right. So you had assisted Caroline Oakley earlier with [Baby D]?

LETBY: Yes.

MYERS: Is this in the same nursery?

LETBY: Yes.

MYERS: And so what's happening here?

LETBY: Caroline is now doing the same for me.

MYERS: All right. If we pull out, please, and go down next to lines 155 and 156. Thank you.

The time now is 02.39, same morning. Can you help us with what's happening here? This is [Baby D]. What's happening?

LETBY: Again, myself and Caroline Oakley are checking and giving medication for [Baby D].

MYERS: And then line 159, please.

MR JUSTICE GOSS: Just pause for a moment.

(Pause)

MR MYERS: 159, if we could look at that, please. What's happening there, Ms Letby?

LETBY: So that's an infusion being started by myself and Caroline Oakley for [Baby D].

MYERS: Is it possible in any way to tell who took what part with an infusion that takes place?

LETBY: It's not, no. The order of signature doesn't have any relevance to who administered.

MYERS: All right. That's at 02.40. Could we look at lines 161 and 162, please? This is about 15 minutes before the collapse at 03.00. What's happening here?

LETBY: So again that's myself and Caroline Oakley giving medication to [Baby D].

MYERS: Right. Do you have any recollection of what was taking place outside what we're looking at on this chart?

LETBY: No, none at all.

MYERS: And do you remember anything of the circumstances of the deterioration that [Baby D] experienced about 15 minutes after that?

LETBY: No. I have some vague recollection of her resuscitation based on reviewing the notes, but --

MYERS: But in your --

LETBY: -- not independently.

MYERS: -- mind now, thinking back, do you have any recollection of what took place round about 3 o'clock?

LETBY: I don't, no.

MYERS: Let's just carry on through to the final collapse.

We can see what you're recorded as doing, Ms Letby, if we go to line 166, first of all. This is at 3 o'clock.

LETBY: entry on the chart for MRE, who's one of your babies, in nursery 1. That actually coincides with the same time that [Baby D] is recorded as having a collapse at 03.00. You don't recall any detail of that?

LETBY: No.

MYERS: All right. Can we go next to tile 174. Keep in mind the third and final collapse is at 03.45. At tile 174, we've got activity at 03.20 in respect of [Baby D].

Can you help us with what that is as far as you can tell from this chart?

LETBY: So myself and Caroline Oakley have started an infusion at 03.20.

MYERS: There's no further involvement with you and [Baby D] before the collapse at 03.45 that we have recorded?

LETBY: No.

MYERS: Do you recall what took place leading up to the collapse at 03.45?

LETBY: No.

MYERS: Do you have a recollection of being in nursery 1 at the time that [Baby D] collapsed at that time?

LETBY: No.

MYERS: Or of Caroline Oakley calling for you to assist with what had happened?

LETBY: I've no memory of that, no.

MYERS: I just want to check a couple of items from the police interview if you'd help us with this. If you have the interviews there, ladies and gentlemen, can we go to interview 1, please, page 2. Down at the bottom of page 2 we can see four lines up the officer asked you:

"Question: Do you remember who the designated nurse was for instance?"

This is talking about [Baby D], and you said no:

"Question: Or which nursery she was in?"

"Answer: No, I believe she was in nursery 1, yes.

"Question: What makes you think that?"

"Answer: Because I remember drawing up the medications in there and I think I did draw up some medications in there."

Do you have recollection of that much, drawing up any medications?

LETBY: Yes.

MYERS: You were asked:

"Question: Why would you be involved with the care of [Baby D] if you were not the designated nurse?

"Answer: Because two people are needed to administer drugs.

"Question: You were involved with the care somewhat but you can't remember being asked to help?

"Answer: No.

"Question: Or being approached by another nurse?

"Answer: No."

Given how often it is that you say nurses may call for help, is there any reason for it to stand out if a nurse has asked you to give him or her help?

LETBY: No.

MYERS: There's no insensitivity intended in the way we deal with this, but so far as your recollections are concerned with [Baby D], is there anything else you recall that bears upon what happened to her?

LETBY: No.

MYERS: Or any further involvement than what we've looked at in these charts?

LETBY: No.

MYERS: All right. With all respect to [Baby D] and her family, if you're ready, and we're able to, Ms Letby, I would like to move next to the next counts on this indictment, 5 and 6, the counts relating to [Baby E] and [Baby F]. That's where we go, ladies and gentlemen.

We will deal with them and look at them in the order we have them in the indictment. Count 5 is [Baby E], and we will deal with [Baby E] first and what relates to [Baby E], and then we'll turn to [Baby F].

[Baby E]. Again, just to remind us, can we put up tile 4, please? We'll go into that, please, Mr Murphy, and actually into the page behind that if we may.

Thank you.

[Baby E] and [Baby F], born on 29 July 2015. So we've moved on a little more than a month from where we were with [Baby D].

LETBY: Yes.

MYERS: We're into July, towards the end of July. [Baby E] born at 17.53, gestation 29 weeks and 5 days. [Baby E] was 1.327 kilograms when born. That was a caesarean delivery. He was the first of the two twins.

Can we go to tile 5, please, just to have a little closer look at the conditions connected with [Baby E] at the time of birth. Just go into that.

Scroll down, please, on to the second page behind this tile, Mr Murphy.

Listed here, Dr Beech records the conditions, the birth conditions, with regard to [Baby E]:

"MCDA twins."

Which we have learnt, though we may not remember immediately, is monochorionic diamniotic, single placenta but their own sacs:

"Reverse end-diastolic flow. Intrauterine growth restricted."

It says oligohydramnios, which means too little amniotic fluid, just to remind us, so there are various matters raised there.

Now, that's the position when [Baby E] was born on 29 July 2015. Count 5 that we're looking at deals with events on the night of 3 August and running over just into the early hours of 4 August, so let's just remind ourselves of that and then we'll turn to your evidence on this, Ms Letby.

Just a summary, and then we'll look at the note.

During the evening of 3 August 2015, during that night shift, [Baby E] bled from his mouth at various points.

We'll look at that shortly.

LETBY: 23.40, there was a serious desaturation and the bleeding continued and there was a period of resuscitative care and

response to the bleeding and the problems that [Baby E] was experiencing, but sadly he died in the early hours the following morning.

Were you the designated nurse for [Baby E], Ms Letby?

LETBY: I was, yes.

MYERS: And you made a note or notes at the conclusion of that, didn't you?

LETBY: Yes.

MYERS: So let's have a look at those and other documents you made, and then we can look at the events and what you say about them.

Could we go to tile 206, please, Mr Murphy? We'll start with the lower right-hand section, please. We need to enlarge that a little bit for those following on the screens. This is an entry at 04.31 on 4 August 2015, so after [Baby E] had died that morning:

"Written in retrospect for care given from 20.00. Emergency equipment checked. Fluids calculated. [Baby E] nursed in an incubator with humidity. IV fluids, Babiven and lipid via long line."

And the insulin that [Baby E] received via the long line:

"Prior to 21.00 feed, 16ml mucky, slightly bile-stained aspirate obtained and discarded. Abdomen soft and non-distended. SHO informed. To omit feed.

"At 22.00 large vomit of fresh blood. 14ml fresh blood aspirate obtained from NG tube. Registrar Harkness attended. Blood gas satisfactory. Blood sugar 10.7mmol. Metronidazole and IV ranitidine commenced and given. Sodium chloride bolus given. Mean BP and observations stable. [Baby E] handling well and active. NG tube on free drainage.

"Further 13ml blood obtained by 23.00. Beginning to desaturate and perfusion poor. Oxygen given via Neopuff initially in 24% incubator oxygen. Toes becoming white and [Baby E] cool to touch. Registrar Harkness present throughout.

"[Baby E] began to decline. 23.40, became bradycardic. Purple band of discolouration over abdomen. Perfusion poor. Capillary refill time 3 secs. Emergency intubation successful and placed on ventilator; see medical notes. Required 100% oxygen. Saturations 80%."

It's got the ventilator details:

"Further saline bolus and morphine bolus given. Second peripheral line sited and used to administer drugs. [Dr C] arrived.

"At 00.36, acute deterioration. Resus commenced as documented: x5 adrenaline, x2 sodium bicarbonate, x1 glucose bolus (inaudible) x1 sodium chloride bolus, x1 20ml/kg O negative blood.

"01.01. Chest compressions no longer required. Further decline. Resus recommenced 01.15 and discontinued at 01.23 when [Baby E] was given to parents.

"[Baby E] was actively bleeding ++ from mouth and nose throughout the resus. 6ml blood was obtained from NG tube."

Then perhaps if we just scroll down, there's an amendment at 05.37 that morning with regard to insulin and a morphine bolus, but then this below that, please, can we look at the bottom entry on the left, below the ones we can see?

4 August 2015, 04.51 under the family communication section:

"Mummy was present at start of shift attending to cares. Visited again approx 22.00. Aware that we had obtained blood from his NG tube and were starting some different medications to treat this. She was updated by Reg Harkness and contained [Baby E]. Informed her that we would contact her if any changes. Once [Baby E] began to deteriorate, midwifery staff were contacted."

Pausing there, I'd like to keep the note open and go to the fluid chart, just to ask you about that, please, Ms Letby. The fluid chart -- I'm going to ask if we look at it in paper, ladies and gentlemen, because then we can compare some numbers. So we'll leave this on the screen, if we may, and the fluid chart is in jury bundle 2, and you'll find it behind divider 5, ladies and gentlemen. The one we need to go to has got the red reference J2720 in the bottom right corner and it might be the last piece of paper in that section, actually, behind divider 5.

LETBY: Would it be possible to have another iPad? This one's gone off.

(Pause)

MYERS: As we look at this chart, I wonder, Mr Murphy, if we could go back to the first page of the notes.

If we scroll up, it's the lower right-hand section of the page where we started on the screens.

We'll go over this in some detail as we go along, Ms Letby, but before we do let me just look at some of the figures we have here. Can you see the paper chart in jury bundle 2?

LETBY: I've got it on the iPad.

MYERS: All right, you've got that. If anyone is following it on the iPad and flitting between the two, it's tile 38 if you want to flit between the two, but it might be easier to have one on paper and one on the iPad.

If we look on the paper chart, can you assist us?

If you look at 21.00 on the right-hand side, can you see that?

LETBY: Yes.

MYERS: We've looked at this a few times, so you know where to go. What does the entry for 21.00 contain? I'm looking in the lower part of the chart, up from the initials and the five or six rows above it. Tell us what that has in it.

LETBY: This is in relation to feeds, enteral feeds. So it says "omitted" in the box, which means that the milk feed was not given, and below that is the aspirate that was obtained at that time.

MYERS: Right. What's the aspirate that was obtained?

LETBY: "16ml mucky."

MYERS: Is that what's written in on the chart in paper?

LETBY: Yes.

MYERS: Whose initials are they at the bottom?

LETBY: Mine.

MYERS: If we look across at your note on the screen, can you tell us if that appears in the note?

LETBY: Yes.

MYERS: What does it say in the note?

LETBY: "16ml mucky, slightly bile-stained aspirate."

MYERS: Before that it says, "Prior to 21.00 feed"?

LETBY: Yes.

MYERS: So those two things refer to the same thing; is that right?

LETBY: The aspirate was obtained prior to giving the milk feed and the feed was omitted as a result of the aspirate.

MYERS: If you look on in the note, the next item in the note after it says the aspirate was obtained and discarded it says:

"Abdomen soft and non-distended. SHO informed to omit feed."

Then:

"At 22.00 large vomit of fresh blood: 14ml fresh blood aspirate obtained from NG tube."

That's what it says in the note. Can you go across to the fluid chart and tell us what we have on there for 22.00?

LETBY: So for 22.00 we have a 15ml aspirate of fresh blood.

MYERS: Right. So we've got at 22.00, coming down, "15ml aspirate fresh blood"; yes?

LETBY: Yes.

MYERS: Can we all see that, ladies and gentlemen? Do you know whose the initials are at the bottom of that?

LETBY: Belinda Simcock.

MYERS: Pausing there, that says "15ml fresh blood" on the chart doesn't it?

LETBY: Yes.

MYERS: Looking in the note it says "14ml fresh blood"?

LETBY: Yes.

MYERS: What's the explanation for that discrepancy, if that's what it is?

LETBY: That's an error on typing. I believe what we've written on the paper chart would be the most accurate reflection.

MYERS: So where it says 14ml in the notes it should say 15?

LETBY: Yes.

MYERS: That's your error?

LETBY: Yes.

MYERS: Is there any reference to vomit for 22.00 on the paper chart?

LETBY: There isn't, no.

MYERS: Would you normally write down if there was a vomit?

LETBY: Ideally, yes.

MYERS: And any reason why you didn't write that down?

LETBY: No.

MYERS: Can you help us with the initials at the bottom of that on the paper chart?

LETBY: So I haven't made these readings at the 22.00 hours.

MYERS: When you say these readings, which ones are you talking about?

LETBY: Um.. So all of them other than the "15ml fresh blood" is my writing. The rest of it is not.

MYERS: Which is your writing?

LETBY: The "15ml fresh blood" in the 22.00.

MYERS: Right. And the rest in that column?

LETBY: That's not my writing.

MYERS: Right. We'll come to a bit more of your account of what happened in a moment, but I just want to deal with what's on these charts before we do so.

Can we scroll down on the note on the screen, please, Mr Murphy. On the screen as we look at that, it charts what happened from a little before 23.00 and onwards. If we look across at the fluid balance chart, can you see there's an entry for 24.00?

LETBY: Yes.

MYERS: Can you help us, who made the entry on the paper chart, what I'm calling the paper chart --

LETBY: Myself.

MYERS: -- chart 2720, that's you?

LETBY: Yes.

MYERS: I'm interested again in the readings in the lower part of the chart dealing with aspirates. Can you tell us what you've noted at that time if it's possible?

LETBY: 6ml blood.

MYERS: 6ml blood on the chart there?

LETBY: Yes.

MYERS: That specific reading doesn't appear in the note, does it?

LETBY: No.

MYERS: Is there any reason why that specific reading doesn't appear in the note?

LETBY: Again, that's just an oversight on documentation.

MYERS: Once we get past 24.00 and we go into 4 August, was there continued blood from [Baby E] --

LETBY: Yes.

MYERS: -- at various intervals?

LETBY: Yes.

MYERS: I just want to go, now we've been over those documents, to a bit more about what happened on that shift and things we've heard about in the evidence.

Could we go to tile 115, please. Can we go into the tile behind that? Can you help us with what -- we can see this is the layout for the evening, but you just tell us what that reminds us or tells you of what the layout was for the responsibilities that evening in nursery 1.

LETBY: So [Baby E] and [Baby F] were the two babies in nursery 1 and I was their allocated nurse.

MYERS: Did you remain the nurse caring for [Baby E] and [Baby F] throughout the whole of the shift?

LETBY: No.

MYERS: Did that change?

LETBY: Yes, so as [Baby E]'s needs increased, Belinda Simcock took over the care of [Baby F].

MYERS: Right. That's Belinda Williamson now?

LETBY: Yes.

MYERS: Belinda Simcock is in nursery there with CE. Was Belinda Simcock a band 6?

LETBY: Yes.

MYERS: She took over with [Baby F], you say, as the evening went on?

LETBY: She did, yes, and she assisted me with [Baby E].

MYERS: Right. Are you able to tell us who those initials were on the chart at 24.00?

LETBY: That's Belinda Simcock.

MYERS: As she was then?

LETBY: Yes.

MYERS: I want to ask you a little more about what we've seen on that paper chart, the fluid chart. We might have it in front of us, ladies and gentlemen, in paper, or if it's not there, I'm going to ask Mr Murphy if he could put up tile 38 on the screen so we can see it there if that helps. Just go into that so we can see it. It's the bottom right-hand corner we're looking at in particular, Mr Murphy. Thank you.

We've seen from your notes, Ms Letby, that [Baby E] was due a feed at 21.00.

LETBY: Yes.

MYERS: Did he get a feed?

LETBY: No.

MYERS: So talk us through, assisted by this, what happened and what leads to the entry "16ml mucky".

LETBY: So prior to any nasogastric feed we would aspirate the NG tube to test the position of the tube. Obviously, here where I have aspirated I've got a large volume of mucky bile-stained aspirate back, which is abnormal.

Therefore the feed has been omitted, which would be usual practice: you would not feed a baby that's had a large aspirate that's mucky.

MYERS: Looking at this chart did he receive no other feeds during that evening?

LETBY: Not by me, no.

MYERS: And not recorded here?

LETBY: No.

MYERS: Having obtained an aspirate like that, did you do anything with it or as a result of that?

LETBY: Yes. So I believe I showed it to Belinda Simcock and we informed the SHO.

MYERS: Why would you have shown it to Belinda Simcock?

LETBY: Because it's an abnormal finding and as a band 5 I would escalate that to a band 6.

MYERS: Right. Was the SHO informed?

LETBY: Yes.

MYERS: Who did that?

LETBY: I don't recall.

MYERS: Do you know which doctor in particular was informed?

LETBY: The SHO.

MYERS: Do you know the name of that doctor?

LETBY: No, I don't.

MYERS: Do you recall from your own knowledge anything that was said to the doctor?

LETBY: Yes. The aspirate was described and the advice given was to omit the feed.

MYERS: Was it described by you, do you mean?

LETBY: I don't recall who spoke to the doctor.

MYERS: Do you know in what way the doctor was informed, by which I mean by someone personally or --

LETBY: No, telephone. Telephone, I believe.

MYERS: By telephone you believe. When you say you believe, is that because you remember it or because it's what you think? Help with us that.

LETBY: No, I do have recollection that we telephoned using the phone in nursery 1 but I couldn't say if that was me or Belinda.

MYERS: Did a doctor come to the unit as a result of being informed about the mucky aspirate?

LETBY: No.

MYERS: You may have said this, but I want to be clear. Was there any advice given or conduct or course to be taken given to you or Belinda by the doctor?

LETBY: Yes, to omit that feed.

MYERS: Did a doctor attend at any point as we carry on that evening?

LETBY: Yes, later on when I got the fresh blood.

MYERS: What time are you talking about when you say "later on when I got the fresh blood"?

LETBY: 22.00.

MYERS: Are you saying a doctor did attend then?

LETBY: The registrar came, yes.

MYERS: Do you remember which registrar that was?

LETBY: Dr Harkness.

MYERS: In the note where you're dealing with 22.00, you describe a vomit of fresh blood:

"14ml of fresh blood aspirate obtained from NG tube."

And you go on to say directly:

"Registrar Harkness attended."

Is 22.00 -- why do you pick the time 22.00 as the time Registrar Harkness attended?

LETBY: Because that's when the fresh blood was noted.

MYERS: Can you be precise about the exact time David Harkness came on to the unit?

LETBY: No.

MYERS: Do you know whether he came on to the unit for any reason other than concerns about fresh blood with [Baby E]?

LETBY: I don't recall.

MYERS: Did he come to the unit just for [Baby E] or was he there for any other reason from what you know? I'm not asking you to guess.

LETBY: I'm not sure.

MYERS: We've heard evidence from [Baby E]'s mother, [Mother of Babies E & F], and she's described events when she came to the unit and she describes that being at 21.00 that evening. So we'll go through this step-by-step, but where [Mother of Babies E & F] is concerned do you recall her coming to the unit that evening?

LETBY: Yes.

MYERS: What time or times do you recall [Mother of Babies E & F] coming to the unit?

LETBY: So I remember [Mother of Babies E & F] was present when I took handover, she then left the unit and, from my memory, she came back down around 22.00.

MYERS: Right. How precise are you in that timing?

LETBY: I've written approximately in my notes, but I would say around 22.00 is accurate.

MYERS: So we can follow which note you're referring to, can we put up tile 206, please. We'll go to that and it's the second page behind the notes, Mr Murphy.

Scroll down to where it says "family communication"

in the bottom left section, please. Thank you.

In the family communication we looked at earlier it says:

"Mummy was present at start of shift attending to cares. Visited again approximately 22.00. Aware that we had obtained blood from his NG tube and were starting some different medications to treat this."

So when you referred to what you recall and what you noted, is that what you're referring to?

LETBY: It is, yes.

MYERS: Right. We can take that down, please, Mr Murphy.

I just wanted to be clear about that.

[Mother of Babies E & F] says that when she came down -- I summarise, forgive me, but these two things come from her evidence -- that [Baby E] was screaming and that there was fresh blood around his mouth. Let's just deal with that. Do you recall [Baby E] screaming, using that word, whilst you were caring for him at any point that evening?

LETBY: No.

MYERS: In particular, do you recall him screaming at 21.00 hours?

LETBY: No.

MYERS: Or for that matter, 22.00 hours?

LETBY: No.

MYERS: Was he screaming at that time?

LETBY: No, he was unsettled at some points but he was not screaming.

MYERS: From your experience, if you're on the nursery, in the neonatal unit, and there's screaming, loud screaming, from a nursery, might you expect to hear that if you're elsewhere on the unit?

LETBY: Yes.

MYERS: Did [Baby E] have blood round his mouth when his mum came down?

LETBY: Not from my recollection, no.

MYERS: Could we put up, please, the diagram which we saw and which [Mother of Babies E & F] drew at J2434, please? This is a diagram that [Mother of Babies E & F] had drawn for the police and the area where she says there was blood is marked we can see hatched on the figure's face. Do you see that?

LETBY: Yes.

MYERS: Was there blood like that when [Mother of Babies E & F] came down round about 21.00, she says, 22.00 you say?

LETBY: Not that I recall, no.

MYERS: What do you recall about what happened when [Mother of Babies E & F] came down?

LETBY: When she came down, [Baby E] was unsettled and she offered containment holding for [Baby E], and shortly after that was when Dr Harkness arrived to review him and she was informed that we'd had some blood back from the NG tube and we were starting some medication to treat that.

MYERS: I'm going to ask you just a repeat a few things with your voice a little louder please. You described her coming down and you made reference to Dr Harkness.

Could you repeat what you were saying about that, please?

LETBY: Dr Harkness was there when [Mother of Babies E & F] came down and she was updated by Dr Harkness about the blood that we had found and the medications that we were then starting to treat that.

MYERS: Do you recall why she'd come down?

LETBY: I don't recall specifically, no.

MYERS: Did she have anything with her when she came down to the neonatal unit?

LETBY: I think she'd brought breast milk down.

MYERS: Expressed breast milk?

LETBY: Yes.

MYERS: Do you recall any conversation about a tube irritating [Baby E] or his throat?

LETBY: No, I don't, no.

MYERS: Did you or anybody send [Mother of Babies E & F] away, in other words tell her to go?

LETBY: No, that's not something we would do on the unit; parents are welcome 24/7.

MYERS: From your recollection, was there blood on [Baby E]'s mouth around 21.00?

LETBY: No.

MYERS: There were various descriptions given of the blood in the evidence and I'm not going to go through all of those, I don't need to, but that's the question I want to ask you: was there blood of any description around his mouth at 21.00?

LETBY: No.

MYERS: What time do you say there would have first been blood in association with [Baby E] that evening?

LETBY: 22.00.

MYERS: 22.00. Which nursery was this in?

LETBY: Nursery 1.

MYERS: How common is it for nurses who don't have a baby in nursery 1 to come into nursery 1?

LETBY: Very common. It's where a lot of the medications are kept and a lot of equipment is kept.

MYERS: I just want to -- we'll have a break shortly with his Lordship's leave. But I'd just like to look in the neonatal review, please, for [Baby E], at page 2, line 33.

Maybe Mr Murphy can assist us by putting this on the screen.

(Pause)

It may be possible to get to this another way, Mr Murphy, in fact.

(Pause)

Sadly not. Ladies and gentlemen, I don't know if you're able to get the paper copies of the neonatal review. Let's have a look that way.

Inevitably, it appears as we get hold of the paper copies. There's no criticism. Thank you for getting us there.

We've got it in stereo now. For those of us with paper copies, could we look down at page 2. And if we get page 2 on the screen, please, Mr Murphy.

Thank you. It's just the last couple of lines I'd be grateful if we go to. Line 33, in fact.

We can see that there. We've been looking at what [Mother of Babies E & F] said took place at about 21.00 hours. Do you recall if you were looking after both twins that evening?

LETBY: I was, yes.

MYERS: Both in nursery 1?

LETBY: Yes.

MYERS: What's happening at 21.13 in nursery 1? Can you help with us that?

LETBY: Myself and Caroline Oakley are giving medication to [Baby F].

MYERS: Right. If we go over the page, please, to page 3, lines 34 and 35, 21.13. What's happening then?

LETBY: This is a continuation of medication being given to [Baby F] by myself and Caroline Oakley.

MYERS: In the same nursery as [Baby E]?

LETBY: Yes.

MYERS: Did Caroline Oakley make any comment about blood on [Baby E]'s face?

LETBY: No.

MYERS: Or [Baby E] screaming or anything like that at this point?

LETBY: No.

MYERS: Or any active bleed ongoing?

LETBY: No.

MR MYERS: Thank you, Mr Murphy, for getting us there.

My Lord, I wonder if that's a time when we could stop for a break.

MR JUSTICE GOSS: Certainly. About 15 minutes. Thank you.

(3.11 pm)

(A short break)

(3.23 pm)

(In the absence of the jury)

MR MYERS: My Lord, before the jury return, may I just raise -- well, two matters. The first one is this: we've run over a little bit the hour that we would normally take after lunchtime, therefore with your Lordship's leave I wonder whether I could conclude around about 4.15.

MR JUSTICE GOSS: Yes.

MR MYERS: I will aim to reach today, to conclude at that time by then.

The other matter is this, and I raise this with appropriate sensitivity, it was brought to my attention that in the course of the evidence, particularly in the last session, there was some reaction to things that were being said in evidence in the court from the public gallery. That is understandable given what we're dealing with and the nature of the case. It's not something I've seen, it's something that has been related to me.

MR JUSTICE GOSS: It's not something I've seen.

MR MYERS: No, but I just --

MR JUSTICE GOSS: I keep a pretty regular eye on it because if I had seen something, I would have said something.

MR MYERS: That is simply enough for me to say that, that's all. Having been raised with me it has caused some concern. If your Lordship is aware of the situation then that gives us great comfort. We understand the pressures, inevitably, and how this must feel but we do ask, through your Lordship, that feelings not be displayed because that can create unfairness if that happens in a number of directions. We leave it with your

Lordship, but I felt obliged to raise it gently and I do. That's all.

MR JUSTICE GOSS: Everyone's heard what you've said and what I've said. We'll just carry on.

Whenever it's convenient and appropriate and most helpful for everyone, Mr Myers.

MR MYERS: Thank you.

(In the presence of the jury)

MR MYERS: Ms Letby, I would like next your assistance for us to look at a passage from the interview -- the second interview dealing with [Baby E]. So if we put the neonatal review to one side or out of the way and, ladies and gentlemen, if we can find the interview bundle, interview bundle 1, and go to the [Baby E] section and go behind divider 2 this time, the second of the interviews, and to page 28. There's a section there I would like to ask you about. So I'll read it through but stop to ask you questions as we go along.

We're going to start just over halfway down the page where the officer says:

"Question: Okay, a statement's been obtained, Lucy, from [Mother of Babies E & F], the mother of [Baby E]. She's given us an account of this night, 3 August 2015, going into the 4th. She says:

"'I expressed some milk and took it to the neonatal ward at around 9 pm.'

"Tell me what you remember about [Mother of Babies E & F] arriving in the ward at around 9 o'clock, Lucy."

And you said:

"Answer: I can't remember, I'd have to look at what I documented."

And you are provided with your note and the officer says:

"Question: So you do recall her arriving on the ward at 9 o'clock?

"Answer: No, not that specific time, no."

So at this point what time do you understand the officer is asking you about?

LETBY: About 21.00.

MYERS: Yes, 9 o'clock?

LETBY: Yes.

MYERS: The officer says:

"Question: Okay, do you recall [Mother of Babies E & F]?

"Answer: Yes.

"Question: The mother of [Baby E] was she? Do you recall if she was expressing milk?

"Answer: I don't remember.

"Question: Is it common for mothers to express milk and bring --

"Answer: Yes.

"Question: She goes on to say:

"'When I arrived [Baby E] was crying and really upset, he had blood coming out of his mouth.'

"Do you recall that, Lucy?"

And what's your response?

LETBY: "No".

MYERS: And by no, what are you saying no to, what suggestion?

LETBY: That [Mother of Babies E & F] arrived at 21.00 to find [Baby E] crying with blood coming out of his mouth.

MYERS: Right. And you said, no, it wasn't happening like that?

LETBY: That's right.

MYERS: What's the officer's next question to you?

LETBY: "Well, tell me about the blood around [Baby E]'s mouth at this time."

MYERS: Have you actually said there was blood coming out of [Baby E]'s mouth around this time?

LETBY: No.

MYERS: You say:

"Answer: I can't remember what time specifically I saw blood or didn't see blood, I don't remember it."

The officer says:

"Question: Okay, so from your notes in your previous interview you commented that [Baby E] had a large amount of fresh blood at 10 o'clock on 3 August. I'll give you a copy, if you just have to look at your notes there Lucy --

"Answer: Yeah.

"Question: -- can you see there at 10 o'clock that you discussed in your first interview" --

MR JUSTICE GOSS: It does say actually "vomit", not "a large amount of blood", "a large vomit".

MR MYERS: Sorry, "a large vomit of fresh blood".

Thank you, my Lord:

"Question: I'll give you a copy if you just have a look at your notes there, Lucy.

"Answer: Yeah.

"Question: Can you see there at 10 o'clock that you discussed in your first interview and you put that the mum is visiting again at 10 o'clock.

"Answer: Yeah.

"Question: You can see that where you have documented that in your notes?

"Answer: Yeah, yeah.

"Question: Do you recall [Mother of Babies E & F] visiting now that you have had chance to look at your notes?

"Answer: Yeah, I remember she came down."

Just to be clear, were you at any point accepting that [Baby E] had blood in his mouth when [Mother of Babies E & F] came down?

LETBY: No.

MYERS: Go over the page. You're asked to give details so far as you can remember of what happened when [Mother of Babies E & F] came down. That's what we have for the large part of that page. But I want to go down and pick up the questioning where the officer says:

"Question: Just confirm the time you've put down there, Lucy, for us that that's happened. This is when you've updated the registrar. You say 22.00.

"Answer: Okay.

"Question: Do you recall, Lucy, when [Mother of Babies E & F] attended the ward that [Baby E] had blood on him?"

And what's your answer?

LETBY: "No, I don't remember."

MYERS: Yes. In particular have you agreed at any point that he had blood on him at 21.00?

LETBY: No.

MYERS: Have you ever suggested he had blood on him at 21.00?

LETBY: No.

MYERS: The officer says:

"Question: Do you have any recollection of [Mother of Babies E & F] arriving and seeing [Baby E], her son, crying and being really upset?

"Answer: I can't remember what [Baby E] was like when she visited.

"Question: Do you recall any conversation with [Mother of Babies E & F] about the blood around [Baby E]'s mouth, Lucy?

"Answer: Not from my memory now, no."

Yes, that's what you say, isn't it?

LETBY: Yes.

MYERS: And have you at any point accepted that there was blood around [Baby E]'s mouth, that you were talking about that with [Mother of Babies E & F]?

LETBY: No.

MYERS: You were asked questions about the containment technique and the next but one question down:

"Question: She said that you said to her it would just be his feed -- his feeding tube irritating his throat and the registrar would be along to see him soon. Do you agree you said that to her, Lucy?"

And what was your response?

LETBY: Sorry, where are we?

MYERS: You see in the margin it says 00.20.48, and you were asked:

"Question: Do you agree that you'd said the feeding tube is irritating his mouth?"

LETBY: No, I don't recall ever saying that.

MYERS: You say, "I don't remember saying that"?

LETBY: No.

MYERS: I was going to ask you, is that something you recall ever saying?

LETBY: No.

MYERS: You were asked about:

"Question: You told her to go back upstairs; do you recall that?"

And your response?

LETBY: No.

MYERS: And had you told her to go back upstairs?

LETBY: No.

MYERS: I want to ask about this:

"Question: She says you told her it would just be his feeding tube irritating his throat. Explain to me what made you believe it was his feeding tube?"

Have you ever accepted in this interview that it was his feeding tube --

LETBY: No.

MYERS: -- that you talked about or said anything about it?

LETBY: No.

MYERS: I would like to move, please, to page 34. I'm looking five lines up from the bottom of page 34, Ms Letby, where the officer says:

"Question: Okay, from the evidence, Lucy, that we've read out to you, you were at [Baby E]'s cot side when his mum [Mother of Babies E & F]'s walked in and when he had blood around his mouth; do you agree with that?"

And what do you say?

LETBY: From [Mother of Babies E & F]'s recollection, yes.

MYERS: Have you ever said he had blood around his mouth when his mum attended?

LETBY: No, I haven't, no.

MYERS: So what are you relying on there when the police are asking you that question?

LETBY: The accuracy of [Mother of Babies E & F]'s recollection.

MYERS: You're asked:

"Question: Do you agree, having spent time looking at them, that the time on your notes is incorrect?"

What do you say?

LETBY: "Yes, I've written an approximation of 22.00."

MYERS: To be clear, when was it that you say you first saw blood?

LETBY: 22.00.

MYERS: Is that the same time as when [Mother of Babies E & F] says she came down at 21.00?

LETBY: No.

MYERS: If we go down a bit further down that page:

"Question: Did you make any attempts to clean up the blood?"

This is after questioning about Registrar Harkness, but first of all, did you make any attempts, Ms Letby, to clean up blood after [Mother of Babies E & F] had come down?

LETBY: Not that I recall, no.

MYERS: Was there any point up to 22.00 when you made any attempts to clean up blood?

LETBY: No, there wasn't blood before that point.

MYERS: You say:

"Answer: I don't remember, but I don't think I would have left him with blood in his mouth."

LETBY: That's right.

MYERS: The officer says:

"Question: Why didn't you escalate it then, Lucy, like you said that you would have done?"

And go over the page, you say:

"Answer: I don't remember that, I don't know whether I did escalate it or not. I thought Belinda Simcock was involved at some point but I don't know at what point I escalated or what I was escalating."

From your recollection what are the occasions that evening when you did raise an issue, either yourself or through someone else, about [Baby E]'s condition?

LETBY: First time was at 21.00.

MYERS: Was there another time?

LETBY: Yes, at 22.00.

MYERS: At 21.00, did that have anything to do with blood?

LETBY: No.

MYERS: What was 21.00 in relation to?

LETBY: It was in relation to the large mucky aspirate that I'd had back. That did not have blood in it.

MYERS: Can we go over the page, please, to page 37. Just looking down at the final questions on this. The bottom of page 37, please. The officer's been asking you questions about the mucky aspirate and the SHO being informed, by your account, and that you said you'd been told to omit the feed. Then the last question by the officer on that page, returning to what we've been looking at:

"Question: Can you explain to me, Lucy, why the bleed which [Mother of Babies E & F], [Baby E]'s mum, witnessed was ignored?"

Have you ever said in this interview that there was a bleed on [Baby E] when his mum had come down?

LETBY: No.

MYERS: And have you ever accepted that you ignored a bleed?

LETBY: No.

MYERS: And did you ignore a bleed?

LETBY: No, I didn't.

MYERS: Did anyone on that unit raise with you the fact that [Baby E] was bleeding and you were doing nothing about it?

LETBY: No.

MYERS: Other members of staff?

LETBY: No.

MYERS: Caroline Oakley, for example?

LETBY: No.

MYERS: Over the page, please, to page 38. Questions about the bleed again and what happened with [Mother of Babies E & F]. I want to get down to the final question at the bottom of the page which stands out alone, it doesn't follow from anything else so I can go straight there. The officer says:

"Question: Is there any explanation you can give, Lucy, as to why the bleed witnessed by [Mother of Babies E & F] was never recorded and why you didn't take any immediate action to seek help from anyone?"

Do you see that?

LETBY: Yes.

MYERS: So far as you recall and record, was there a bleed witnessed by [Mother of Babies E & F]?

LETBY: No.

MYERS: In particular, was there a bleed at 21.00?

LETBY: No.

MYERS: Did you fail to record a bleed?

LETBY: No.

MYERS: Did you ignore it?

LETBY: No.

MYERS: What did you do when eventually there was, on your account, a bleed?

LETBY: I contacted the registrar and escalated to Belinda Simcock.

MYERS: Which registrar was that?

LETBY: Dr Harkness.

MYERS: Thank you for dealing with that.

Do you recall after Dr Harkness had come, I'm not asking minute by minute, but do you recall the way in which events went from that time on, from about 22.00 on in general terms?

LETBY: Yes.

MYERS: Could we put up tile 38, please, which is the fluid chart. Thank you. Just go behind that again to look at the chart down in the bottom right-hand corner, please.

When we come to the record here it says "15ml fresh blood". What was your opinion when you saw that?

LETBY: That was very concerning.

MYERS: Right. It's something you routinely encountered on the unit, something like that?

LETBY: No.

MYERS: Between the 15ml of fresh blood and what's recorded here as "6ml blood" at 24.00, to the best of your recollection was there any further blood at that stage between those two times or do we just have what's recorded?

LETBY: No, just what's recorded, I think.

MYERS: In your note you have said that:

"At 23.40, [Baby E] began to decline, became bradycardic and there was a purple band of discolouration over the abdomen and perfusion was poor."

LETBY: Yes.

MYERS: Can you describe in any more detail what it was that you saw in terms of discolouration?

LETBY: So [Baby E]'s stomach was becoming more and more distended and with that distension, there was a red sort of horizontal banding across his abdomen.

MYERS: Can you actually, if you can recall, show us what the mean by horizontal banding across his abdomen?

LETBY: It was just... It was just across where his umbilical cord was, just around that area --

MYERS: All right.

LETBY: -- just a line.

MYERS: Was the colour anywhere else?

LETBY: No.

MYERS: Spreading anywhere else?

LETBY: No, it was just on the abdomen.

MYERS: It was just on the abdomen. Was the colour moving round other parts of the body?

LETBY: No, it wasn't, no.

MYERS: Did you have any opinion as to what that was?

LETBY: I wondered if he was bleeding into his abdomen and his abdomen was becoming distended with the discolouration.

MYERS: The note says:

"[Baby E] began to decline. 23.40, became bradycardic, purple band of discolouration over abdomen."

Is that the order in which events went and the order in which you saw the discolouration?

LETBY: Yes.

MYERS: Do you recall how long that discolouration lasted for?

LETBY: It stayed there throughout.

MYERS: Go to your notes, please, at tile 206. It's the note on the second page. If we can enlarge the upper part of that, please, Mr Murphy.

We can see what I have just referred to about, "[Baby E] began to decline at 23.40"; can you see that?

LETBY: Yes.

MYERS: And the description that you gave. We've got the reference to [Dr C] arriving and then, 00.36, acute deterioration.

LETBY: Yes.

MYERS: Can you describe to us, beyond what we see in the notes, what happened in terms of [Baby E]'s deterioration or are you unable to do so?

LETBY: At this point he had been intubated and he just declined from there, his observations declined and he was actively bleeding.

MYERS: When you say "actively bleeding", that might mean different things to different people. Can you help us understand what you mean by actively bleeding?

LETBY: So there was blood coming into the free drainage pot that was on the NG tube.

MYERS: What's that, an open -- what is a free drainage pot or the tube or the pot on the free drainage tube?

LETBY: It's a small pot that's connected to the end of the NG tube so when volume comes out you can see it then filling the pot.

MYERS: And that was just coming out, was it?

LETBY: It was.

MYERS: As events moved on, did [Baby E] bleed any more than that?

LETBY: Yes, he was bleeding from his mouth and his nose.

MYERS: Were chest compressions performed upon him?

LETBY: Yes.

MYERS: What happened when that took place?

LETBY: He was actively bleeding from his mouth and nose.

MYERS: Did you assist during the resuscitation?

LETBY: Yes.

MYERS: Were his parents there?

LETBY: Yes.

MYERS: Is this something you wanted to happen?

LETBY: No.

MYERS: Had you done something to make this happen?

LETBY: No.

MYERS: Were you there when [Baby E] died?

LETBY: Yes.

MYERS: Did you have any part to play in what happened after [Baby E] died?

LETBY: Yes.

MYERS: What did you do?

LETBY: As designated nurse it was my role to care for [Baby E] and for the parents after his death.

MYERS: Did you do that to the best of your ability?

LETBY: I did, yes.

MYERS: What sort of things did you do?

LETBY: I bathed [Baby E].

MYERS: All right. Did he have clothes?

LETBY: No, he didn't have clothes at that point, so I found him a gown from the unit to be dressed in.

MYERS: We know that sometimes photographs are taken; did that happen?

LETBY: Yes.

MYERS: Did his family want that?

LETBY: Yes.

MYERS: What about handprints and footprints?

LETBY: Yes.

MYERS: Did you do that?

LETBY: Yes.

MYERS: Did you have any help with it?

LETBY: Yes.

MYERS: Who helped you?

LETBY: Belinda Simcock.

MYERS: Were [Baby F] or [Baby E] ever given a teddy bear or a cuddly toy or anything like that?

LETBY: The memory boxes that we make come with two small teddy bears, one was given to [Baby E] and one given to [Baby F].

MYERS: Were any photographs taken of that?

LETBY: Yes.

MYERS: Who took those photographs?

LETBY: I did.

MYERS: Were they set up or staged in any particular way?

LETBY: No.

MYERS: Who wanted the photographs taking?

LETBY: The parents.

MYERS: You'd looked after [Baby E] on this evening. Did you continue to look after [Baby F] afterwards?

LETBY: On this night?

MYERS: Going forwards after this night.

LETBY: Yes.

MYERS: Was there anything considered -- did anyone consider it strange that you should look after [Baby F] after you'd been looking after [Baby E]?

LETBY: No.

MYERS: How did you feel about what had happened to [Baby E]?

LETBY: I found [Baby E]'s death very traumatic. I'd never seen a baby bleed in that way before.

MYERS: I'm going to turn to [Baby F] and again I do so understanding what we're dealing with here. I'll pause a moment.

(Pause)

We'll look at [Baby F]'s birth details. In many ways, of course, they reflect [Baby E]'s because the lads were twins, but I'm going to ask Mr Murphy if we could look at some of the matters in the exhibits relating to [Baby E], which is on a different part of the system -- sorry, to [Baby F], my apologies. To [Baby F].

We remind ourselves about the details for [Baby F].

That's at tile 3, please, Mr Murphy.

Are you all right to carry on, Ms Letby?

LETBY: Yes.

MYERS: All right. Thank you.

Date of birth, of course, 29 June. [Baby F] was the second twin and the time of birth is 17.54 and [Baby F] was 1.434 kilograms in weight.

Can we look at tile 3, a little bit -- just a little bit further down. I can see in the writing, "Issues in pregnancy". Just so we have this, we see IUGR; is that intrauterine growth restriction?

LETBY: Yes.

MYERS: TTTS, is that --

LETBY: Twin-to-twin transfusion syndrome.

MYERS: That's where identical twins share a placenta; is that right?

LETBY: Yes.

MYERS: There may be issues with the blood flow between them?

LETBY: That's right.

MYERS: Then mild -- we can see the first matter, "Mild polyhydramnios"; do you know what that means?

LETBY: Too much amniotic fluid.

MYERS: Around the baby during pregnancy?

LETBY: Yes.

MYERS: All right. With [Baby F], count 6 on this indictment, we're looking at issues in relation to his blood sugar, his blood glucose in particular, and insulin. Are you okay to carry on?

LETBY: Yes.

MYERS: Just by way of summary, so we're all in the right place for this, we'll have a look at the chart in a moment to remind ourselves, but what we have is from 5 August 2015, from about 01.55 until about 19.00 that same day, there were issues with [Baby F]'s blood glucose, it was too low, persistently low.

LETBY: Yes.

MYERS: And you understand what this is to do with, don't you --

LETBY: Yes.

MYERS: -- the allegation?

LETBY: Yes.

MYERS: There was an analysis of a blood sample that was taken at 17.56 on the 5th, so later into that day. That returned the following reading: c-peptide, 4,657 picomoles per litre -- I'll start again: insulin, 4,657 picomoles per litre; C-peptide, 169 picomoles. Insulin, 4,657, C-peptide, 169. And as we've heard from the experts, that ratio is way out.

LETBY: Yes.

MYERS: The insulin is far higher than it should be for that level of C-peptide. And you understand that allegation, don't you?

LETBY: Yes.

MYERS: The first thing we'll do is just to remind ourselves from the chart what it is that happened. Then I'll ask you to talk us through some of the exhibits as far as we can this afternoon, Ms Letby; all right?

LETBY: Yes.

MYERS: Let's take time to look at the chart and remind ourselves what that showed. Apologies for my voice, but it's just to remind us what we dealt with.

Can we go to divider 6, please, in jury bundle 2, ladies and gentlemen. Ms Letby, have you got a copy of that, jury bundle 2? Have a look at that, please.

If we go to the back or towards the back of that, ladies and gentlemen, you might remember there's a chart which we have from Professor Hindmarsh that looks like this (indicating). I'm holding it up. It's the table of blood glucose readings.

LETBY: I don't have a copy.

(Pause)

MYERS: J3146. Sorry, I4261.

(Pause)

MR JUSTICE GOSS: I know exactly what you're referring to when it's held up, but it's not in that section.

MR MYERS: The only other place I could think it is in is in [Baby M]'s section.

MR JUSTICE GOSS: It's jury bundle 1.

MR MYERS: Ms Letby, can you see it on the screen?

LETBY: Yes.

MYERS: So we've either got it in paper behind divider 6 in jury bundle 1 or on the screens. This is just to refresh our memories about what we have.

Let's go from 5 August 2015 at 01.54. We're not going to go to all the individual tiles. 01.54, which is about 30 minutes or so after a TPN bag is hung that we're going to go and have a look at. Blood glucose is at 0.8. What would the normal range of blood glucose be for a neonate, Ms Letby?

LETBY: Between 2 and 2.5, more than that.

MYERS: So certainly 0.8 is low?

LETBY: Yes.

MYERS: At 02.55, the blood glucose is 2.3, is that still low?

LETBY: I can't remember whether it's more than 2 or more than 2.5 in the protocol at that time.

MYERS: All right. 04.02, it's 1.9; that's low?

LETBY: It is, yes.

MYERS: At 5 o'clock we have a reading of 2.9.

LETBY: That's within normal range.

MYERS: Yes. But we'll see when we come to look at it in the evidence, that followed something that had happened with dextrose.

MR JUSTICE GOSS: At 04.20.

MR MYERS: Your Lordship's there, yes.

MR JUSTICE GOSS: I've written it down: 10% dextrose, 3ml at 04.20. That's what Professor Hindmarsh said would account for the rise.

MR MYERS: We'll go back to that, but that's right. That's what happens there. In fact, for that matter, there's a 10% dextrose just before the rise or the figure at 02.55. We'll have a look at that as well.

MR JUSTICE GOSS: Yes, at 02.05, there was a previous one.

MR MYERS: Thank you, my Lord.

08.09, 1.7. 10 o'clock on the 5th, down to 1.3. It says:

"Off intravenous fluid administration as cannula problems."

You might remember there was an issue with the cannula tissinging and a change of TPN and we'll come to that. That was a little later.

11.46, 1.4. That's still low, isn't it?

LETBY: Yes.

MYERS: 12.00, 2.4. We know some time around there, there's a bag change, but we'll go to the documents in due course.

14.00, 1.9. 16.00, 1.9. 18.00, 1.9. Those are all low, aren't they?

LETBY: Yes.

MYERS: At 18.55, nothing directly next to that, though there's a tile. The TPN was stopped then at 18.55, the TPN was stopped and in fact a dextrose solution was run and at 19.00 the blood glucose is up to 2.5.

LETBY: Yes.

MYERS: That's back into the normal range, isn't it?

LETBY: Yes.

MYERS: And 21.17, 4.1. That's normal, isn't it?

LETBY: Yes.

MYERS: Right. Before we come to look at your part in [Baby F]'s care, let me ask you this, Ms Letby: did you administer insulin to [Baby F]?

LETBY: No.

MYERS: Did you interfere with any of the TPN bags so that could happen?

LETBY: No, I didn't.

MYERS: Do you know why the insulin/C-peptide ratio was as high as it was for [Baby F] when that sample was taken at 17.56?

LETBY: No.

MYERS: If [Baby F] did receive insulin he should not have received, does that have anything to do with you?

LETBY: No, it doesn't.

MYERS: Do you have any idea how that will have happened if that is what has happened?

LETBY: No.

MYERS: Can we just look at the layout, please, for the shift where that first TPN bag that's followed by the 0.8 blood glucose reading is hung? That's at tile 100.

Let's see who's on that shift at the start of this.

This is on the [Baby F] sequence, tile 100.

Shift leader, Belinda Simcock. In terms of nursing staff, this is. Designated nurse, [Nurse A].

She's looking after [Baby F] that night; is that right?

LETBY: Yes.

MYERS: Do you have any independent recollection of that night outside the records and charts that we have?

LETBY: Not really, no.

MYERS: Right. You were on the shift, so is Sophie Ellis, so were Valerie Thomas and Cheryl Cuthbertson-Taylor. Can we have a look at the layout of who was where? That's at tile 101.

This is the evening after the early morning with [Baby E], isn't it?

LETBY: Yes.

MYERS: And you and [Nurse A] are both working in nursery 2?

LETBY: Yes.

MYERS: Do you recall whether you'd cared for [Baby F] at other times in the period before this whilst he'd been on the neonatal unit?

LETBY: The night that [Baby E] died.

MYERS: All right. As we've heard before, this isn't a memory test.

MR JUSTICE GOSS: She's already said she was the designated nurse for both of them to begin with.

MR MYERS: Yes, on that night.

MR JUSTICE GOSS: [Baby E] and [Baby F]. But when [Baby E] became ill, she relinquished that role.

MR MYERS: So certainly the night before you played a part?

LETBY: Yes.

MYERS: You were looking after [Baby F]?

LETBY: In the early part of the shift, yes.

MYERS: Can we go back, actually, to the intensive care chart at page 3200. J3200. This isn't in the sequence, but just to have a look at your care with [Baby F]. Thank you.

We're going to look at 3200 into 3201. This is an intensive care chart for [Baby F] on 1 August 2015.

Just look down at 20.00 hours on that chart. Just enlarge down in the bottom right-hand corner, please.

This isn't in our materials at this point up to now because it hasn't been on the dates we're looking at, but let's look at this now.

So for the evening shift of 1 August at 20.00, who is performing the various cares for [Baby F]?

LETBY: That's myself.

MYERS: 20.00, 21.00?

LETBY: Yes.

MYERS: 22?

LETBY: Yes.

MYERS: 23?

LETBY: Yes.

MYERS: 24?

LETBY: Yes.

MYERS: Are you caring for him?

LETBY: Yes.

MYERS: Did you want him to be all right?

LETBY: Yes.

MYERS: Looking after him?

LETBY: Yes.

MYERS: Can we go over the page, please, to 3201, which continues into the early hours of the morning of 2 August. At 01.00 whose signature is that?

LETBY: They're my signatures.

MYERS: Where do we see your signature running through until?

LETBY: The last one is at 07.00.

MYERS: Right. So that covers the period, in fact, of the evening shift, the night shift from the 1st into 2 August?

LETBY: Yes.

MYERS: That would be you looking after [Baby F]; yes?

LETBY: Yes.

MYERS: And if we scroll down a little further down on this chart, let's pick it up again now going into the evening shift of the 2nd because that was the morning on the 2nd. Can we scroll down to the lower part of the chart, please, Mr Murphy?

If we pick it up again -- you're there until 07.00 with [Baby F], and now at 20.00 who's that looking after him on the IC care chart on the 2nd?

LETBY: That's myself again.

MYERS: Right. Can we just go over to page 3202? Carry on through the night of that shift into the following morning into 3

August. Who is that caring for him until 07.00, the end of that shift?

LETBY: Again, that's myself.

MYERS: As you said and his Lordship reminded us, on the night of the 3rd into the 4th, you were also caring for the twins, so you were caring for [Baby F] then?

LETBY: Yes.

MYERS: And we then dealt with the events into the 5th with the blood glucose, we're going to come back to that. Before we do, could we go, please, to page 3008?

Could we look at the left-hand side, please, Mr Murphy, first? 11.57. You understand the allegation, Ms Letby, is that on or around 5 August you tried to kill [Baby F]. You understand that's the allegation?

LETBY: Yes.

MYERS: This is a note by you at 11.57 on 8 August written for care of [Baby F]. How did you want him to be as you were looking after him on this occasion?

LETBY: I wanted him to be well.

MYERS: How did you want him to be?

LETBY: What do you mean, sorry?

MYERS: Did you want to care for him?

LETBY: Yes.

MYERS: Did you want to hurt him?

LETBY: No.

MYERS: Did you do anything to, I hate to put it indelicately, try and finish off anything that had been started anywhere else --

LETBY: No.

MYERS: -- which is one of the themes in the allegation you are facing?

LETBY: No.

MYERS: Is that something you would do, go back and finish it?

LETBY: No.

MYERS: Are you doing that here?

LETBY: No.

MYERS: You cared for him on 8 August, we see that in the note there. Can we go to 9 August, please, at page 3013?

This is a note at 15.30 on 9 August:

"Written for care given from 07.45 to present. Emergency equipment checked."

Who's written this note, Ms Letby?

LETBY: That's me.

MYERS: "Fluids calculated. [Baby F] nursed in an incubator with humidity. Observations within normal range.

Remains on Optiflow. Flow reduced."

You describe that:

"In air, well tolerated. Continues with two-hourly feeds."

Who would have been feeding him?

LETBY: Me.

MYERS: "EBM via nasogastric tube [that's expressed breast milk]. Minimal aspirates obtained. Abdomen appears full but soft. Urine passed and watery milk stool.

Handling well. Medications given as prescribed via long line."

It describes the Babiven infusion and then just below that 15.33:

"No further changes [at 19.54, apologies]. On Optiflow. Prongs positional, manages well when not in situ."

Can we just go below that to the bottom of the page.

You read that out for us, Ms Letby, what it is you're doing here on the family communication, 15.33.

LETBY: You want me to read that out?

MYERS: Yes, please.

LETBY: I've written:

"Parents resident on unit. Daddy has carried out cares this morning. Parents have been home for a few hours to make funeral arrangements for sibling, [Baby E]. Now on unit with other family members."

MYERS: And the note finishes over the page if we could, please.

Scroll down, I'd be grateful, Mr Murphy, to the next page.

Could you finish that, Ms Letby?

LETBY: "Parents carried out cares and both had cuddles with [Baby F], happy that he has opened both eyes for the first time today."

MYERS: All right. [Baby F] was discharged from the neonatal unit to another hospital on 13 August; yes?

LETBY: Yes.

MYERS: Did you do anything in this period we're looking at, let's move on from 5 August, anything to hurt [Baby F]?

LETBY: No.

MYERS: Or would you have wanted that?

LETBY: No.

MYERS: When you did have anything to do with him, what was your aim?

LETBY: For him -- to care for him, to get him well enough to go home.

MR MYERS: What I'd do next, if I could, is to turn now to what's on the TPN prescriptions and the signatures, but I wonder whether, my Lord, this is an appropriate point to stop and to turn to that on Monday (sic) morning because there's quite a lot of detail in them as we go through the bags.

MR JUSTICE GOSS: We're not going to get through it all this afternoon, so it's best to do it in one piece, but that will not be until Thursday of next week.

MR MYERS: Yes, of course.

MR JUSTICE GOSS: It's another two-day week next week.

Can I just check, because I think I had put this in jury bundle 1, I'd actually got that table that Professor Hindmarsh prepared at the front of section 5, but I think you all have it in section 6; is that right?

Yes, you did. I could see from the colour of the documents.

Is there anything in section 5? There is, yes, there's some blood sugar -- there's another table, the blood sugar readings, aren't there, for [Baby L]? Is that right? So you've got nothing in 5? I put them all in 5 because that was the next one and I've got nothing in 6.

MR MYERS: The table was put together with the [Baby F] table. I've got that bit right. But then I put them both in a different place, so I'll have to hunt them down.

MR JUSTICE GOSS: What I'm going to ask is for Mr Astbury to prepare an index for JB1 for next Thursday so we can all have the documents in the same place. I don't mind which you put it in, but we'll deal with that on Thursday.

MR MYERS: Thank you, my Lord.

MR JUSTICE GOSS: Thank you, Mr Myers. All right.

Well, a long break now until Thursday of next week, ladies and gentlemen. Of course you will adhere to the directions that I have given you: no communication with anyone about anything to do with this case or anyone to do with this case and no research about anything to do with this case. We'll continue on Thursday of next week. 10.30. Thank you very much indeed.

(In the absence of the jury)

MR JUSTICE GOSS: Thank you, Mr Astbury. If you could just do that. Speak, obviously, to Mr Maher or someone just so that you agree that that's the best places for them, for the various documents. Thank you.

Right. Now then, are you in a position to deal with the email that I was sent in relation to the request for the release of exhibits to the media from Ms Moritz, Judith Moritz?

MR MYERS: I haven't yet had the opportunity to speak to Mr Johnson about it, so there's not a --

MR JUSTICE GOSS: I'll see what Mr Johnson has to say.

MR MYERS: There may not be any issue.

MR JOHNSON: First of all, I'm happy to do it with Ms Letby sitting there.

MR JUSTICE GOSS: I think otherwise the reconfiguration and the wasted time...

MR JOHNSON: The position, my Lord, is that at an early stage of the proceedings various of the handwritten Post-it notes were released to the press and we didn't bring it to your Lordship's attention, but a national newspaper with a very wide internet circulation published one of them with the name [Dr A] clearly legible.

LETBY: soon as we found that out, we contacted the representative of the organisation and that person very swiftly had it removed from the website, so we're not complaining about that.

MR JUSTICE GOSS: The only problem is that once things get on -- once things are out, they're out. They get taken down, but they're there.

MR JOHNSON: Absolutely. But the problem is that because a lot of these documents are so detailed, we are -- when I say we, I'm referring to the CPS -- not happy to take on the responsibility of giving what we think is a sanitised version, which subsequently turns out not to be and the press relying on what they're given by us.

So in this restricted class of documents, and there are only a few of them because the press have had most of them already, there are still one or two, which are the ones that are referred to by Judith Moritz, so far as they are concerned they're so busy that we just don't feel comfortable taking the risk. So subject to any further representations that her organisation may wish to make to those who deal with these sorts of things within the CPS, so far as the trial team are concerned we would -- putting it bluntly, we're not happy at the prospect of putting either the trial at risk or putting the identity of one of these witnesses at risk in light of the orders that your Lordship has made, and whilst we understand --

MR JUSTICE GOSS: It wasn't actually me, the orders that have been made, certain orders were made before. The orders that are made in these proceedings. It doesn't matter. Anyway, there are two specific exhibits that are identified --

MR JOHNSON: Yes.

MR JUSTICE GOSS: -- KFL-4 and ELM-4 and what you're saying essentially is that the redaction process would be such that -- well, there may be a mistake made, essentially.

MR JOHNSON: Exactly.

MR JUSTICE GOSS: Well, I'm not going to get involved in it at this stage, but I do think that there could be some discussion between the media officer at the CPS, or whoever is dealt with, to see whether there's a way through or not in relation to this.

MR JOHNSON: Yes. We don't want to get involved in it at all, frankly, because we've got enough to be worried about.

MR JUSTICE GOSS: Exactly. There are some photographs in the last paragraph that are referred to as well.

MR MYERS: I'll address your Lordship with regard to that unless Mr Johnson has anything to add.

MR JOHNSON: No.

MR MYERS: Yes, first of all, we recognise why the media are interested in this and we understand that and we understand the responsible reporting which has taken place. This is not in any way critical of the intentions or the approach.

The particular matter that engages us is the request to have copies of the images of the house and the interior of the house. There are two problems with that. The first one is the practical one that it's something we would choose to do on instruction, the view we take with regard to that, and that's not something we're in a position to deal with at the moment, we can't take instructions from Ms Letby as to what she would say. Therefore, so far as our instructions are concerned, we'd be reticent in the extreme in assenting to something which has not been discussed with the defendant, so we can't do that.

But there's a second matter and that is that two of the photographs of the interior have images that contain themselves redacted images of children who are connected to the defendant. Their faces are blanked out but the relationship between them and the defendant has been raised in evidence, she said who those people would be, and there's at least one large note on the noticeboard which we saw which makes reference to "Number 1 godmother".

So we would have some concerns as to any issue that could lead in any unwanted way or any intrusive way into a focusing upon young people who have a connection with the defendant arising

out of what is visible on that noticeboard and pictures elsewhere, particularly, for example, with those in the locality who know -- and it may be said, well, they know already, so what's the harm, but we would prudently wish not to engage in an experiment to find out what the harm would be, but we just perceive a possibility of a problem.

Moreover, on that image in particular there are some telephone numbers and contact details which, insofar as it's presented to the jury, creates no issue but is material that, we would observe, should not readily be put into the public domain. We don't know who they would all relate to or where those numbers would go.

I suppose they could be redacted if the decision were taken to pass this on, but we share some of the concerns of the prosecution with this material so far as the images are concerned where they display personal details of other people, whether that is in numbers and letters or in images, albeit partially redacted and our concerns are heightened because some of those are young people.

So my observation, my Lord, is, first, we're not in a position in any event to assent to any of this, but even if we were, we would have some concerns in relation to the matters I have identified.

MR JUSTICE GOSS: Thank you. Ms Moritz, what I'm proposing to do is -- you've heard what has been said -- I'm proposing to defer this. I'm not shutting anyone out from making any further representations, but may I suggest that there is communication between you or whoever represents you and the CPS in relation to the particular exhibits you want. You've heard what Mr Myers has said about the potential identification of people in relation to some of the photographs relating to the house and perhaps there could be some discussion about perhaps there's some photographs but not others, so to speak. See what I mean?

To be honest, yes, it is the house but it's not as though it's a crime scene or anything like that, it's rather, if I may say so, potentially prurient interest rather than anything else, whereas the actual exhibits that are directly referable to alleged offences are of more public interest.

I'm not shutting anyone out, but I think there has to be a sort of -- there has been a very responsible approach, and I'm not suggesting there wouldn't be a responsible approach, but I'm suggesting that may be a way forward and we'll revisit it next week or later on in the trial. But thank you very much for raising it in the way that you have done, it's entirely appropriate if I may say so.

MEMBER OF THE PRESS: (Inaudible) there is a clear distinction.

MR JUSTICE GOSS: Absolutely. I can understand -- I've drawn the distinction as well in relation to the first two exhibits. Continue to liaise. Thank you very much indeed.

Is there anything else? Mr Johnson, Mr Myers?

Thank you very much.

Ms Letby, if you remain where you are as usual.

Could I ask everyone else to leave the court? That is the end of the proceedings for today and indeed this week until Thursday of next week.

(4.21 pm)

(The hearing adjourned until 10.30 am on Thursday, 11 May 2023)